

PathScribe by ForMedrix AI

User Guide

Clinical Reporting System

For Pathologists and Residents
v0.9.0 | June 2026 | Confidential -- PathScribe AI

Table of Contents

Table of Contents	2
1. Getting Started	6
1.1 Logging In	6
1.2 Biometric Setup	6
1.3 The Navigation Bar	6
2. The Home Screen	6
3. The Worklist	7
3.1 Status Tiles	8
3.2 Table Columns	8
3.3 Sorting	9
3.4 Pool Cases	9
3.5 Delegated Cases	10
4. Opening a Case	10
4.1 Navigation within a Case	10
4.2 Patient History	10
5. Diagnostic Information	10
6. Synoptic Reporting & AI Copilot	11
6.1 AI Copilot Suggestions	11
6.2 Accepting and Rejecting Suggestions	11
6.3 Protocol Change Review (After Microscopic)	11
6.4 Coding	12

7. Computational Sidecar	12
7.1 Worklist Computational Icons	12
7.2 Synoptic Report -- Computational Tab	12
7.3 Cancelling a Pending Order	13
7.4 Ordering Additional Tests	13
8. Report Preview	15
9. Case Finalisation	15
9.1 Pre-Finalisation Review	15
9.2 After Finalisation	15
10. Case Delegation	16
10.1 Delegating a Case	16
10.2 Receiving a Delegated Case	16
11. Case Search	16
11.1 Smart Identifier Box	17
11.2 Search Filters	17
12. Internal Messaging	18
12.1 Reading Messages	19
12.2 Composing	20
12.3 Request Informal Review	22
13. Voice Commands	23
14. Your Profile	24
16. Validation Studies	25

17. My Contribution	25
17.1 Overview	26
17.2 Productivity, Quality, and AI Contribution	26
18. Additional Case Tools	27
18.1 Case Team	27
18.2 Flag Manager	28
18.3 Report Sequencer	29
18.4 Comments	30
18.5 Internal Notes	30
18.6 Code Manager	31
18.7 Launch EMR	32
18.8 Patient History	33
19. Audit Log	35
20. Top Navigation Bar -- Additional Elements	35
20.1 Clinical Links	35
20.2 Enhancement Request	36
20.3 QA / Testing Feedback	37
20.4 Voice Toggle Indicator	38
Quick Reference	39
Appendix A -- Colour & Visual Language Reference	41
A.1 Case Status Indicators (Worklist dots)	41
A.2 Case Flags	41
A.3 TAT Progress Bars	41

A.4 Severity Badges (Quality Tab)	41
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Index	42
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1. Getting Started

1.1 Logging In

Open Chrome or Edge and navigate to your PathScribe by ForMedrix AI address. Enter your email and password on the ForMedrix login screen and click Sign In. Credentials are issued by your system administrator.

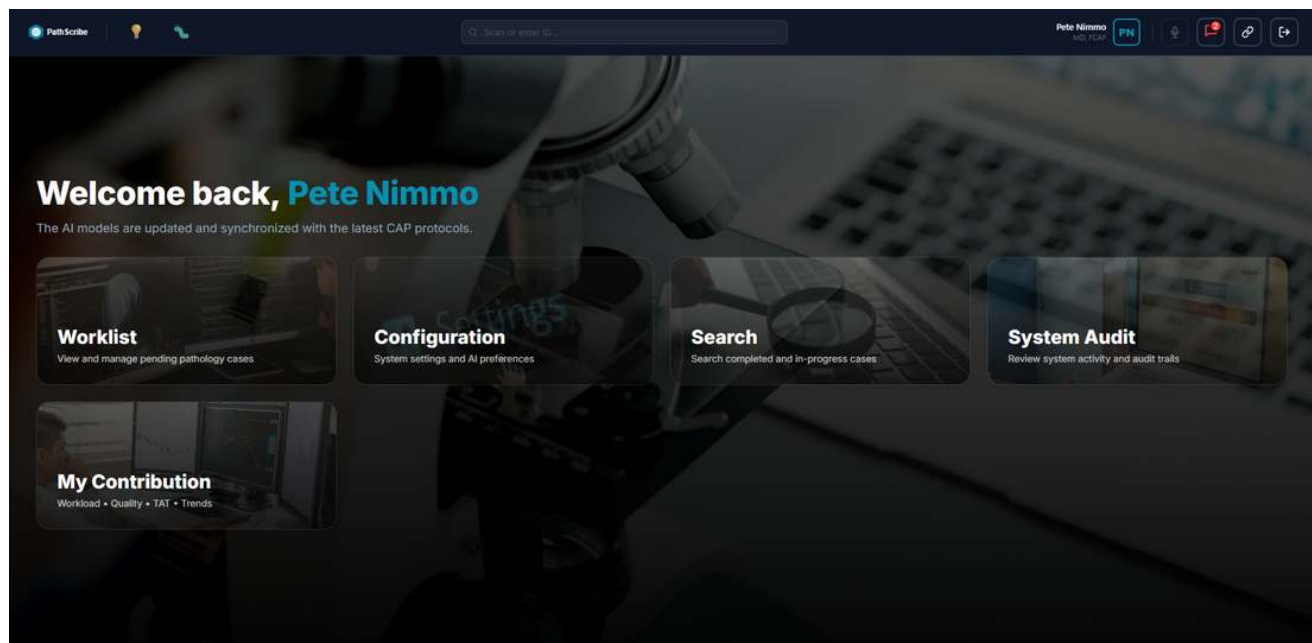
1.2 Biometric Setup

On first login PathScribe may prompt you through the Biometric Setup Wizard (fingerprint or face recognition). You may skip this; it can be completed any time from your profile. Once enrolled, biometric sign-out replaces the password prompt in the Pre-Finalisation Review.

1.3 The Navigation Bar

Element	Action
PathScribe logo	Click anywhere to return to the Home screen.
Enhancement Request	Submit feature requests or bug reports.
Voice toggle	Enable/disable hands-free voice commands.
Messages	Open the internal messaging drawer.
Clinical Links	Quick access to CAP protocols, WHO Classification, and PathologyOutlines.
Your name / initials badge	Profile card -- User Guide, Admin Guide (if admin), System Information.
Sign Out (arrow)	Log out of PathScribe.

2. The Home Screen



Home screen -- Worklist, Configuration, Search, System Audit, My Contribution tiles.

Tile	What it opens
Worklist	Your active case queue -- the primary daily view.

Tile	What it opens
Contribution	Personal productivity dashboard: workload, quality, and TAT metrics.
Search	Cross-institutional case search with advanced filters.
System Audit	Audit trail of all system events (admin-visible).
My Contribution	Detailed personal metrics by specialty, TAT trend, and quality.
Configuration	System settings, user management, template builder (admin role).

3. The Worklist

The Worklist is your primary clinical view. STAT / critical cases are pinned at the top under a red Urgent banner; remaining cases follow in sort order.

The screenshot displays the PathScribe Worklist interface. At the top, there's a navigation bar with the PathScribe logo, a search bar, and a user profile for Pete Nimmo. Below the navigation bar, the 'Worklist' tab is active. The 'Active Cases' section shows a summary of case counts: LIS CASES (12), OUTREACH (5), URGENT (3), POOL CASES (3), DELEGATED TO ME (0), URGENT (4), IN PROGRESS (4), NEEDS REVIEW (1), AMENDED (1), COMPLETED TODAY (0), DRAFT (5), and FINALIZING (0). The main table lists cases with columns: Case, Patient, MRN, Sex, DOB (Age), Specimen(s), Accession, Physician, Flag(s), and Status. Cases are categorized into 'URGENT' (red banner) and 'ALL CASES' (blue banner). Urgent cases include S26-4402-COLON-RES, S26-4403, S26-4406, and S26-4408. All cases include S26-4401-BX-001, S26-4404, S26-4405, S26-4407, and S26-4409. Each case row shows patient details, specimen information, accession number, physician name, and various flags indicating case status and actions.

Active Cases -- default view, LIS-sourced cases with rich flag chips and computational icons.

Active Cases

LIS CASES 12 URGENT 5 OUTREACH 5 POOL CASES 3 DELEGATED TO ME 0 URGENT 2 IN PROGRESS 1 NEEDS REVIEW 0 AMENDED 0 COMPLETED TODAY 0 DRAFT 4 FINALIZING 0

Case	Patient	MRN	Sex	DOB (Age)	Specimen(s)	Accession	Physician	Flag(s)	Status
026-0002	Okafor, Patricia	200002	F	09/03/1965	A: Right lower lobe lobectomy B: Station 7 subcarinal lymph	06/02/2026	Mr. Andrew Pearce		
026-0004	Kovacs, Sandra	200004	F	07/19/1982	A: Left total mastectomy B: Left axillary sentinel lymph	06/08/2026	Dr. Rachel Kim		
026-0001	Ashford, Robert	200001	M	04/22/1959	A: Right hemicolectomy B: Resected lymph nodes, s... C: Appendix	06/02/2026	Mr. James Caldwell		
026-0003	Marchetti, David	200003	M	01/08/1962	A: Radical prostatectomy B: Right pelvic lymph nodes C: Left pelvic lymph nodes	06/08/2026	Mr. Simon Hartley		
026-0005	Nakamura, Grace	200005	F	03/12/1988	A: Total thyroidectomy B: Right central compartment	06/08/2026	Dr. Thomas Walsh		
026-0012	Fernandez, Alicia	500001	F	05/17/1992	A: Cervical cone biopsy (L.L... B: Endocervical curettage	06/02/2026	Dr. Jennifer Moss		
026-0013	Briggs, Harold	500002	M	01/30/1955	A: Skin excision -- left fore...	06/08/2026	Dr. Carol Simmons		
026-0014	Castellano, Yvonne	500003	F	08/11/1974	A: Right breast core needle	06/08/2026	Dr. Martin Osei		

Active Cases -- LIS Cases and Outreach counted and grouped separately, alongside Urgent and Pool sections.

NOTE

Outreach cases (accession IDs starting with O26-) are tracked as a distinct count from LIS-sourced cases on this screen, reflecting that PathScribe owns the full reporting lifecycle for them rather than the host LIS.

3.1 Status Tiles

Tile	Colour	What it shows
Total Cases	Grey	Your full non-pool caseload. Always a summary -- not a filter.
Pool Cases	Indigo	Unassigned cases in the shared pool. Click a pool case to open the Pool Claim workflow.
Delegated to Me	Cyan	Cases routed to you by another pathologist for review or second opinion.
Critical	Red	STAT-priority or critically flagged cases.
In Progress	Blue	Cases actively being worked on in the reporting workflow.
Needs Review	Amber	Cases with an AI-generated synoptic draft awaiting your sign-off.
Amended	Purple	Finalised reports re-opened for amendment.
Completed Today	Green	Reports finalised today.

3.2 Table Columns

Column	Contents
(dot)	Pulsing red dot = STAT/critical. No dot = routine.
Case	Accession number. Red = critical, cyan = routine. Click to open.
Patient	Last, First name (PHI). Sortable alphabetically.

Column	Contents
MRN	Medical Record Number (PHI-protected).
Sex	M / F / O. Sortable.
DOB (Age)	MM/DD/YYYY with compact age label (e.g. 52y).
Specimen(s)	Up to 3 specimen label chips. Hover for full description.
Accession Date	Date specimen received in the laboratory. Sortable.
Physician	Requesting/submitting physician. Sortable.
Flag(s)	Coloured flag chips. Solid border = case-level. Dashed = specimen-level. Hover for name.
Status	Coloured dot: Draft, In Progress, Finalizing, Finalized, Amended.

3.3 Sorting

Click any column header to sort. Click again to reverse direction. PathScribe supports multi-column sort -- up to three sort keys simultaneously. Sort preferences persist across sessions.

Computational Data Quick Peek: hover or click the computational icon on a worklist row to see molecular/IHC results inline -- without leaving the worklist or opening the full case.

The screenshot shows the PathScribe interface with a top navigation bar and a sidebar. The main area displays a table of 'Active Cases' with columns for Case ID, Patient, MRN, Sex, DOB (Age), and Specimen(s). A 'Quick Peek' modal is open, showing molecular profiling results for a specific case. The modal includes a 'Molecular Profiling' section with a 'Preliminary' status and a table of results.

Case ID	Patient	MRN	Sex	DOB (Age)	Specimen(s)
S26-4402-COLON-RES	Jackson, Robert	100002	M	08/22/1959 (54y)	A: Sigmoid colon B: Apical lymph node
S26-4403	Williams, Helen	100003	F	11/05/1963 (62y)	A: Right upper lobe B: Station 4R mediastinal lymph nodes C: Station 7 subcarinal lymph nodes
S26-4406	Anderson, Ruth	100006	F	01/28/1968 (56y)	A: Left breast core
S26-4408	Davis, Carol	100008	F	07/03/1965 (59y)	A: Right breast core B: Right axillary core
S26-4401-BX-001	Thompson, Grace	100001	F	03/14/1974 (52y)	A: Left breast core B: Left axillary core
S26-4404	Martinez, David	100004	M	05/30/1955 (69y)	A: Prostate biopsy B: Prostate biopsy C: Prostate biopsy
S26-4405	Taylor, Susan	100005	F	09/12/1978 (46y)	A: Right breast core

Molecular Profiling
Comprehensive molecular profiling panel -- five result from LIS

Gene	Result
KRAS	p.G12C mutation detected (PRELIMINARY)
EGFR	Wild type
ALK	Negative
ROS1	Negative
PD-L1 TPS	45% (22C3 assay)

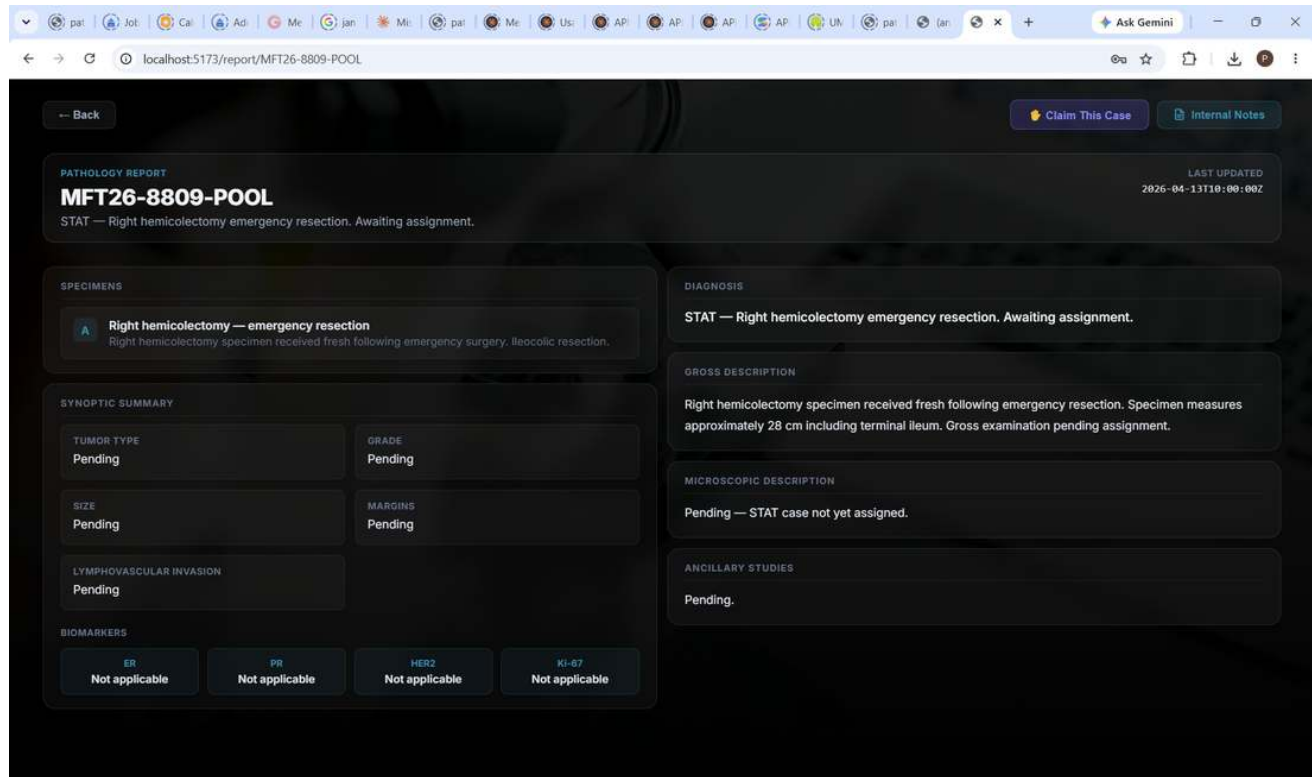
status note: Comprehensive NGS panel in progress -- KRASG12C targeted therapy eligibility pending confirmation

Resulted Jun 12, 2026, 7:34 AM

Quick Peek -- molecular results shown inline from the worklist row.

3.4 Pool Cases

Pool cases are unassigned cases visible to all pathologists at your institution. Click a pool case row to open a read-only preview of the case -- specimens, synoptic summary, and diagnosis as far as they've been entered -- with Claim This Case and Internal Notes available even before you claim it. Click Claim This Case to assign it to yourself.



Pool case preview -- read-only until claimed.

3.5 Delegated Cases

When another pathologist delegates a case to you, it appears in the Delegated to Me tile. Open the case and review the delegation note. You may accept the delegation or return the case with a comment.

4. Opening a Case

Click any worklist row to open the case. The case layout has three panels:

Panel	Contents
Left	Patient demographics, order details, clinical indication, requesting physician, and submitting client.
Centre	Diagnostic workspace: gross description, microscopic description, ancillary studies.
Right	Synoptic reporting form, AI Copilot suggestions, and the Computational Sidecar tab.

4.1 Navigation within a Case

Specimen tabs -- multi-specimen cases show lettered tabs (A, B, C...) at the top of the right panel. Breadcrumb navigation -- click any crumb to return to that level. Previous / Next Case -- use the arrow buttons in the case header bar (or voice commands).

4.2 Patient History

Click the Patient History link in the left panel to open the Patient History modal. This shows prior cases, relevant diagnoses, and clinical history for the patient. Dismiss with the close button, Esc, or click outside the modal.

5. Diagnostic Information

The centre panel contains three autosaving rich-text fields. The AI Copilot reads these in real time and generates synoptic suggestions as you type.

Field	Contents
Gross Description	Macroscopic findings -- specimen dimensions, tumour measurements, margin distances, and macroscopic appearance.
Microscopic Description	Histological findings -- tumour type, grade, invasion, margins, lymph nodes.
Ancillary Studies	IHC, FISH, molecular, flow cytometry, and any other supplementary results.

TIP

Enter the gross description first -- the AI begins generating synoptic suggestions immediately. Complete or refine microscopic and ancillary text to trigger further suggestion updates.

6. Synoptic Reporting & AI Copilot

PathScribe uses CAP-compliant, RCPATH, and Custom synoptic templates. The template displayed matches the specimen type and selected protocol.

6.1 AI Copilot Suggestions

Badge colour	Confidence	Meaning
Green	$\geq 85\%$	High confidence -- strongly supported by diagnostic text.
Amber	50-84%	Moderate confidence -- review the source citation carefully.
Red	$< 50\%$	Low confidence -- treat as a starting point only; verify independently.

6.2 Accepting and Rejecting Suggestions

Action	How
Accept a suggestion	Click the tick icon -- applied to the field immediately.
Reject a suggestion	Click the cross icon and enter your own value.
Accept all	Click Accept All at the top of the synoptic panel.

IMPORTANT

AI suggestions are advisory only. You as the pathologist are responsible for reviewing and verifying every field before finalising. Confidence scores reflect computational certainty, not clinical accuracy.

6.3 Protocol Change Review (After Microscopic)

When you save a microscopic description, PathScribe AI re-evaluates whether the synoptic protocol assigned at gross is still appropriate.

NEW in v0.9.0

The Protocol Change Review modal shows each proposed change with: the current protocol and the proposed replacement side by side; the AI's clinical reasoning from the microscopic text; a confidence score for the proposal; a checkbox to accept or reject each individual change. Select the changes you want to apply, or deselect any you want to keep as-is. Click Apply Changes to commit your selections. Click Keep existing protocols to dismiss without making changes. All decisions are recorded in the audit trail.

6.4 Coding

As synoptic fields are completed, PathScribe automatically suggests ICD-10 and SNOMED CT codes. Review and confirm, add manual codes, or override suggestions in the Coding panel.

7. Computational Sidecar

The Computational Sidecar brings live laboratory results -- IHC, molecular, FISH, cytogenetics -- directly into your PathScribe workflow.

7.1 Worklist Computational Icons

Icon colour	Status	What it means
Teal	Pending	Test ordered. No result received yet.
Amber	Preliminary	Interim result available. Final result still pending.
Green	Final	Final result received. No immediate clinical action required.
Red	Final -- Actionable	Final result received. Review required -- clinical action may be needed.

7.2 Synoptic Report -- Computational Tab

Inside the Synoptic Report, click the Computational tab to open the full results panel. This shows specimen-grouped results with: result data table, status badge, specimen badge, AI concordance indicator, and AI extraction badge.

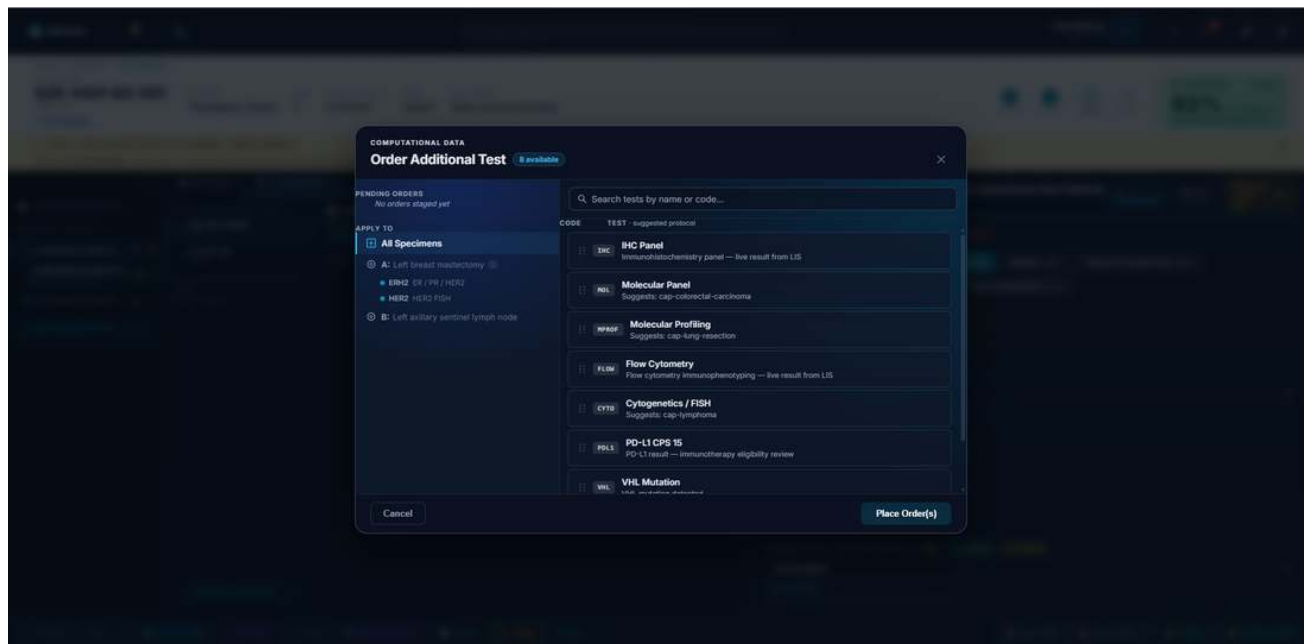
A full case view -- Computational tab open alongside the synoptic checklist, AI confidence scores with Confirm/Override, and the full case toolbar (Launch EMR, Delegate, Team, Request Review, History, Flags, Codes) along the bottom.

7.3 Cancelling a Pending Order

Click the trash icon beside a Pending test in the navigator. Read the confirmation dialog carefully -- if the order originated in the LIS, you must also cancel it there.

7.4 Ordering Additional Tests

Click + Order additional test at the bottom of the Computational navigator. Stage tests for one or more specimens, then click Place N Orders to submit.



Order Additional Test -- search by name or code, apply to one or all specimens.

TIP

You can stage tests for multiple specimens before submitting -- change the Apply To selector between batches and accumulate all orders before clicking Place Orders.

8. Report Preview

Click Preview Report in the case header to open the formatted print-ready report.

UPDATED -- June 2026

The Report Preview pane now shows the complete report -- demographics, specimens, synoptic summary, and sign-off, in addition to the AI-narrative sections (Diagnosis, Gross, Microscopic, Ancillary). Previously, only the AI-narrative sections rendered here; the rest of a case's data never appeared in this pane even though it was part of the template.

Control	Function
Page size selector	Choose A4, US Letter, US Legal, A3, or B5. Your preference is saved.
Margin controls	Adjust top, right, bottom, left margins in mm. Saved per session.
Ruler	Shows margin boundaries above the document.
Print button	Generates the final formatted PDF of the report. If PDF generation is temporarily unavailable, falls back to your browser's print dialogue.

9. Case Finalisation

When all required synoptic fields are complete, click Finalise. Finalisation is a three-step gate:

Step	What happens
1 -- Validation	PathScribe checks all required fields. Missing fields are highlighted. The gate will not advance until all required fields are filled.
2 -- AI Triage Review	Fields the AI flagged as low-confidence (below the configured threshold) are shown one at a time. Review and confirm, override, or skip each item. Keyboard shortcuts: Space/Right Arrow = Confirm, O = Override, S = Skip, Esc = Cancel.
3 -- Pre-Finalisation Review	Full-screen two-pane review before credentials are collected (see below).

9.1 Pre-Finalisation Review

NEW in v0.9.0

The Pre-Finalisation Review replaces the previous direct-to-password confirmation. It is mandatory for all sign-outs.

- **Left pane -- Specimen list:** All specimens and their synoptic reports in transmission order. Drag specimens to reorder them. Click Exclude on any synoptic to remove it from this transmission (it remains in PathScribe as a draft for a future addendum).
- **Right pane -- Report preview:** live, formatted preview of the complete report output as it will be transmitted to the LIS -- the full report, not only the AI-narrative sections (see Section 8 note). Updates in real time as you reorder or exclude synoptics.
- **Signing panel:** Sign using biometric (fingerprint or face, if enrolled) or your login password. This constitutes your clinical authorisation for the transmission and is recorded in the audit trail.

9.2 After Finalisation

Once finalised, the case status changes to Finalized and the report is transmitted to the LIS (Co-Pilot mode) or locked in PathScribe (Standalone mode). The report is locked for editing. Amendments require re-opening by an administrator.

IMPORTANT

Resident counter-sign workflow: If you are a Resident, your report is submitted for attending review rather than directly finalised. The attending must counter-sign to complete the process.

10. Case Delegation

Delegation routes a case to a colleague for peer review, specialist opinion, second opinion, or subspecialty referral.

10.1 Delegating a Case

Step	Action
1	Open the case and click the Delegate button in the case header.
2	Select the recipient from the staff directory (search by name or subspecialty).
3	Choose the Delegation Type (Peer Review, Second Opinion, Subspecialty Referral, MDT Discussion).
4	Add a note explaining what you need from the recipient.
5	Click Confirm Delegation -- the recipient receives a notification and the case appears in their Delegated to Me tile.

10.2 Receiving a Delegated Case

Click the Delegated to Me tile. Open the case -- the delegation note and requesting pathologist are shown at the top of the right panel. Complete your review and either return the case or accept full ownership.

11. Case Search

Case Search (Home -> Search tile) finds cases across your institution using any combination of filters. All active filters combine with AND logic.

Case Search
Search across cases, specimens, diagnoses, and clinical codes

Showing cases with accession May 22, 2026 -- Jun 21, 2026

Case	Patient	MRN	Sex	DOB (Age)	Specimen(s)	Accession	Physician	Flag(s)	Status
URGENT: HFH526-1005-FS	Washington, George	300005	M	05/18/1971	Pancreatic head mass	06/20/2026	Dr. Mazen Iskandar	P STAT -- Intraoperative Frozen Section P OH Awaiting Result	
MFT26-8806-CR-STAT	Blackwood, Edward	200006	M	06/30/1952	Sigmoid colectomy	06/20/2026	Mr. Peter Thornton	P STAT -- Rush Processing P Tumor Perforation -- pT4	
S26-4402-COLON-RES	Jackson, Robert	100002	M	08/22/1959	Sigmoid colon resection Apical lymph node	06/19/2026	Dr. Michael Torres	P STAT -- Rush Processing P Oncology Awaiting Report	
S26-4403	Williams, Helen	100003	F	11/05/1963	Right upper lobe lobectomy Station A1 mediastinal lymph node Station 7 subcarinal lymph node	06/19/2026	Dr. James Park	P STAT -- Rush Processing P STAT -- Rush Processing P Thoracic MDT -- Fri 09-09	
S26-4406	Anderson, Ruth	100006	F	01/28/1968	Left breast core needle biopsy	06/19/2026	Dr. Patricia Moore	P STAT -- Rush Processing P Breast Section Pending	
S26-4408	Davis, Carol	100008	F	07/02/1965	Right breast mastectomy Right axillary contents	06/19/2026	Dr. Sarah Chen	P STAT -- Rush Processing P Oncology Treatment on Hold P BRCA Pathologic Variant	
S26-4482-RENAL	Halloran, Victor	200103	M	11/03/1964	Partial nephrectomy Surgical margin shave	06/19/2026	Dr. Nathan Briggs	P STAT -- Rush Processing P Urology MDT	
ALL CASES: DEMO-RB-01	Bishop, Eleanor	DEMO100001	F	03/14/1974	Right breast core biopsy	06/19/2026	Dr. Pamela Winters		
DEMO-RB-02	Monroe, Celia	DEMO100002	F	11/07/1978	Left breast core biopsy	06/19/2026	Dr. Sandra Okafor	P BRCA4 AC	
DEMO-RB-03	Hale, Martin	DEMO100003	M	05/22/1959	Sigmoid colon polyp Ascending colon polyp	06/19/2026	Dr. James Fowler		

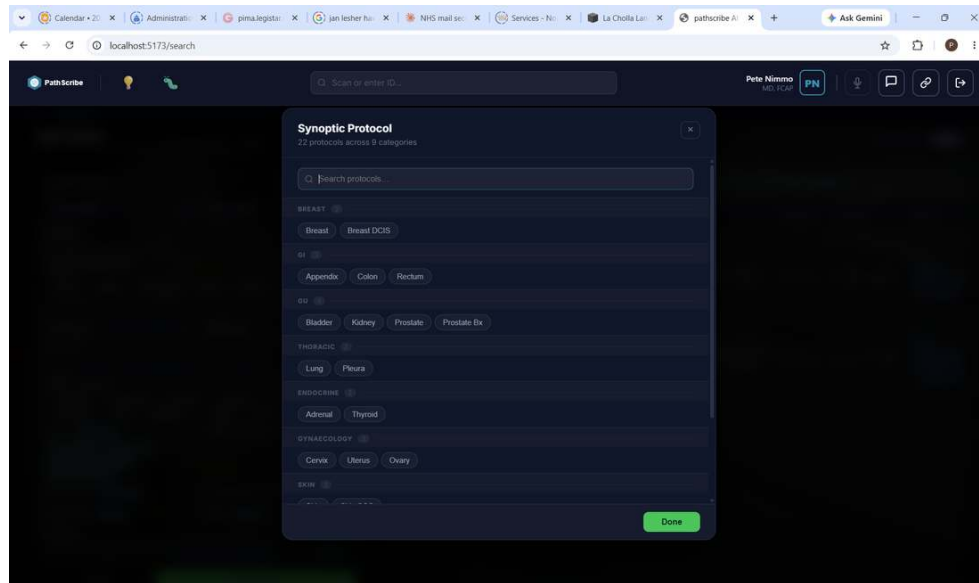
Case Search -- full filter sidebar and result list, including flag chips and case status.

11.1 Smart Identifier Box

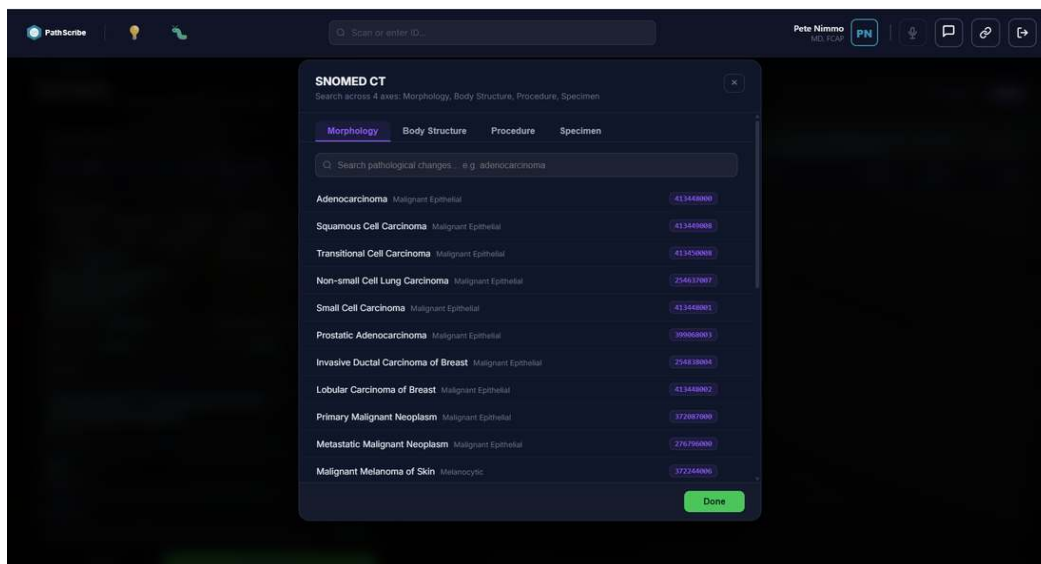
What you type	Detection	Badge colour
S26-4200	Accession number (matches your lab format)	Purple
Smith, Jane or Jane Smith	Patient name	Green
123456 (numeric only)	MRN / Hospital ID	Cyan
abc123 (mixed)	All fields -- searches simultaneously	Amber

11.2 Search Filters

Filter	Notes
Accession Date	Date window. Quick-pick: 7d, 30d, 90d, 1yr. Default is last 30 days.
Status	Grossed, Awaiting Micro, Finalizing, Completed. Multi-select.
Priority	Routine or STAT.
Flags	Click Browse. Multi-select. Flags are additive (OR logic within the flag filter).
Synoptic Protocol	Filter by CAP or institutional protocol. Click Browse for the full library.
Pathologist	Search by surname or hospital. Multiple pathologists can be selected.
Specimen Type	Multiple specimens allowed.
Diagnosis	Free-text terms. Multiple terms allowed. Press Enter or + to add.
SNOMED CT	Four axes: Morphology, Body Structure, Procedure, Specimen.
ICD Codes	Search across all active coding systems (ICD-10, ICD-11, ICD-O).



Synoptic Protocol browse modal -- 22 protocols across 9 categories.



SNOMED CT browse modal -- Morphology, Body Structure, Procedure, Specimen tabs.

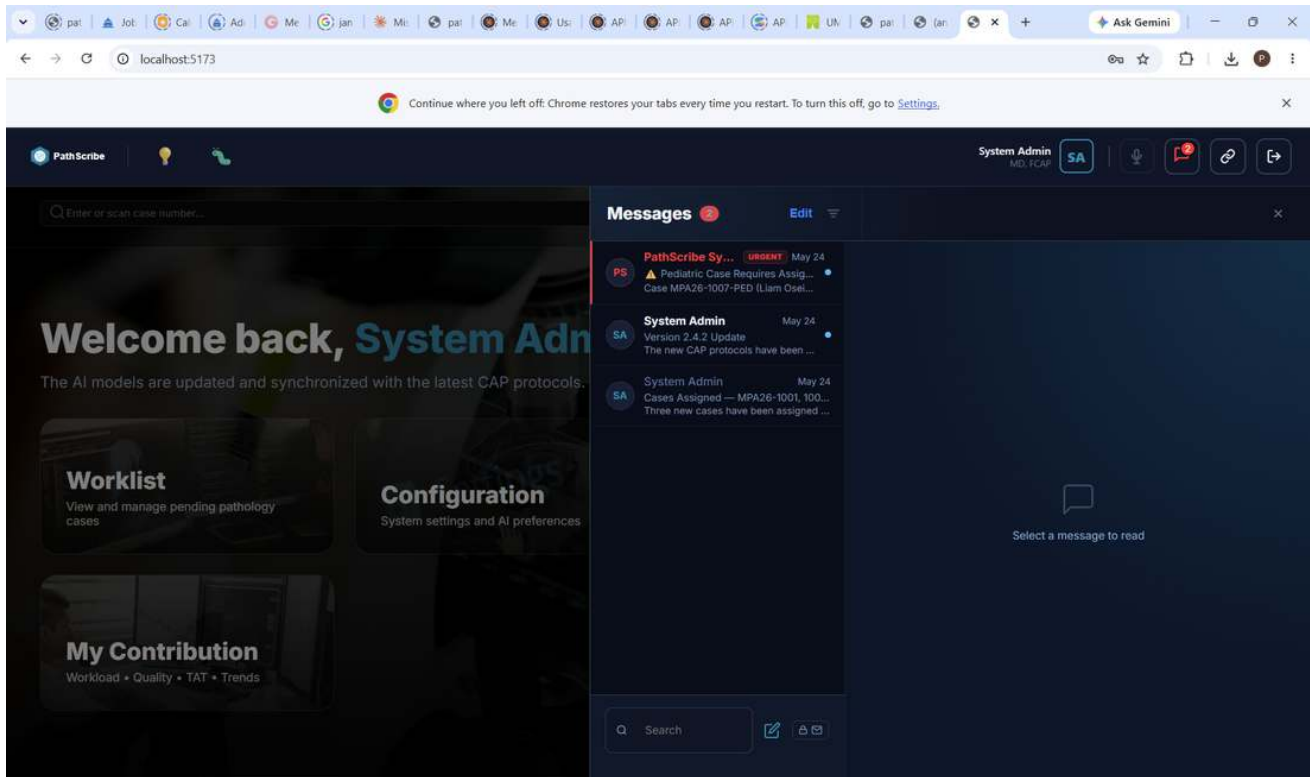
Flags, Pathologist, Specimen Type, Attending Physician, Client, and Computational Flags all use the same Browse-modal pattern -- a searchable, multi-select list with a Done button.

TIP

Save a "Daily STAT" search with a 7-day window and STAT priority for instant morning triage.

12. Internal Messaging

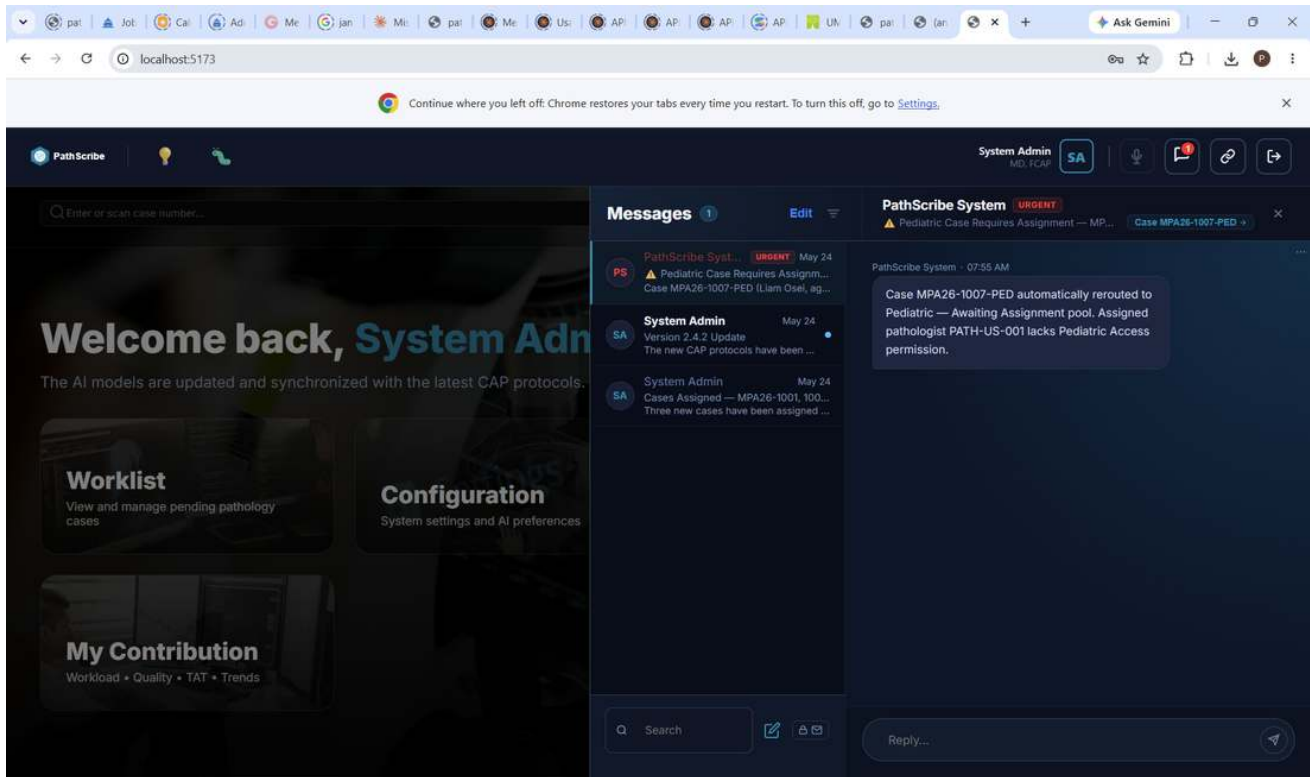
PathScribe's internal messaging system is for secure lab communication. Click the speech bubble icon in the navigation bar to open the messaging drawer.



Messages drawer -- list on the left, selected thread on the right.

12.1 Reading Messages

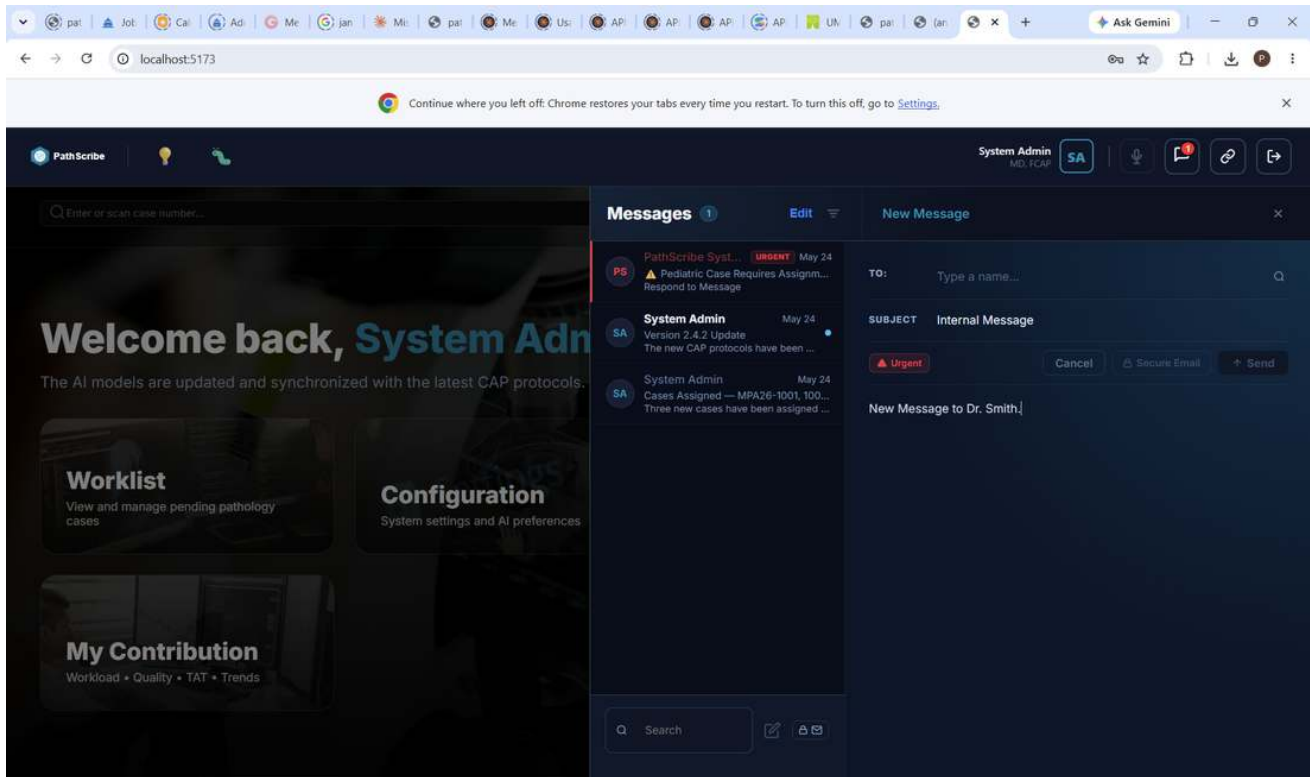
Messages are listed in the left panel. Unread messages are bold with a blue dot. Urgent messages appear highlighted in red. Click any conversation to read the full thread.



Reading a system message -- here, an automated Paediatric Access Control routing notice with a direct case link (Section 2.4).

12.2 Composing

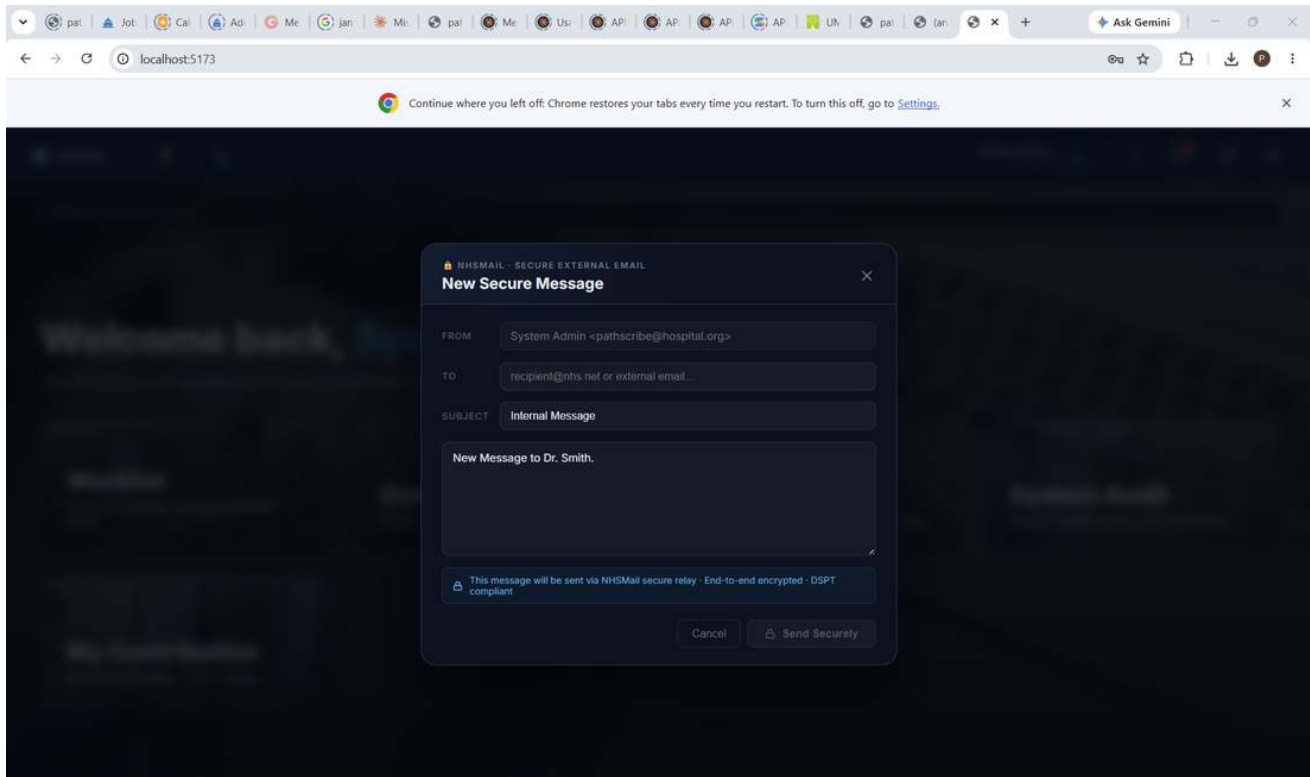
Click the compose icon at the bottom of the message list. Type a recipient name in the To: field, enter a subject, write the body, and toggle Urgent if required.



New Message -- To, Subject, Urgent toggle, and a separate Secure Email option alongside Send.

SECURE EMAIL

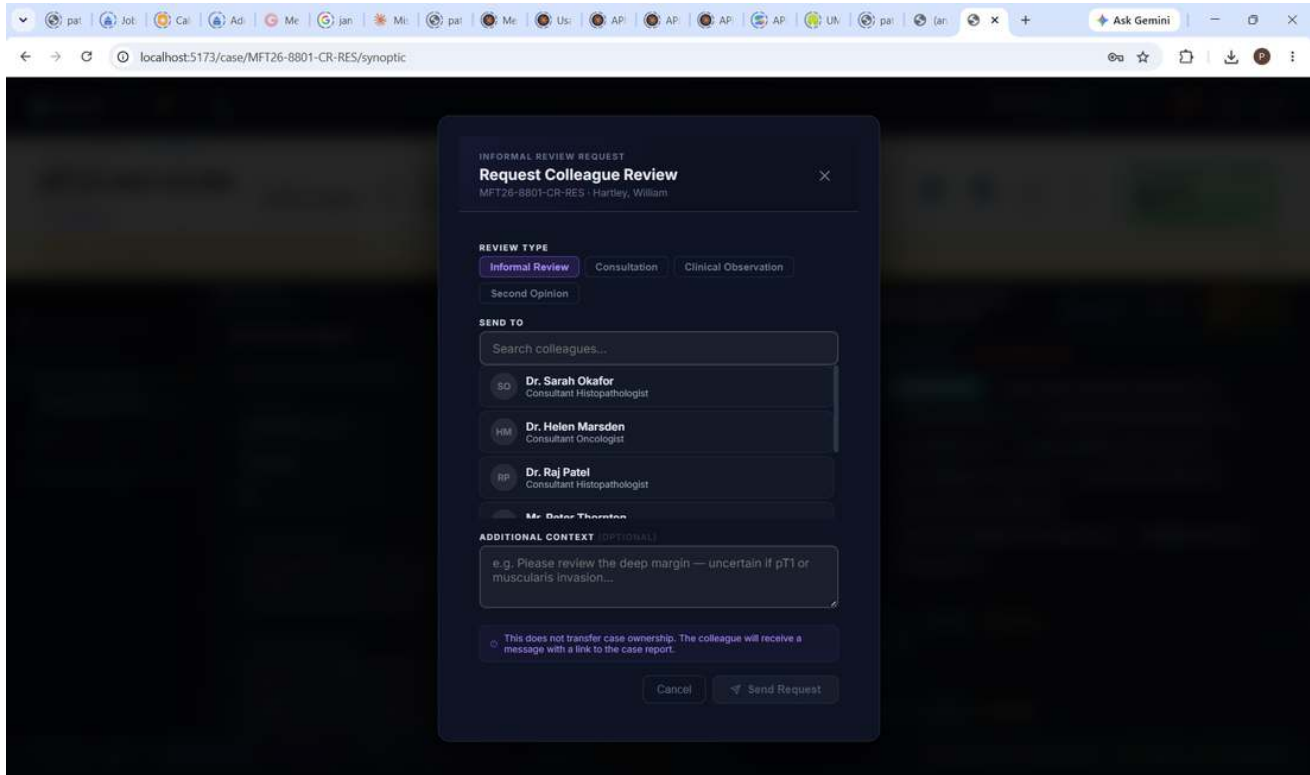
Secure Email sends externally via your institution's secure mail relay (e.g. NHSMail), end-to-end encrypted and DSPT-compliant -- distinct from a regular internal Send, which stays inside PathScribe.



New Secure Message -- routed via NHSMail secure relay, end-to-end encrypted, DSPT compliant.

12.3 Request Informal Review

To request an informal review from a colleague without transferring case ownership, open the case and use the Request Review option in the case header. Select a Review Type (Informal Review, Consultation, Second Opinion, Clinical Observation), search for and select a colleague, optionally add context, and send.



Request Colleague Review -- review type, colleague search, optional context.

NOTE

Request Informal Review does not transfer case ownership. It sends a message only. Use Case Delegation (Section 10) to formally transfer a case.

13. Voice Commands

PathScribe is fully voice-enabled -- every action available via mouse or keyboard can be triggered by voice. Click the microphone icon to activate. The system is context-aware: the same phrase does different things on different screens.

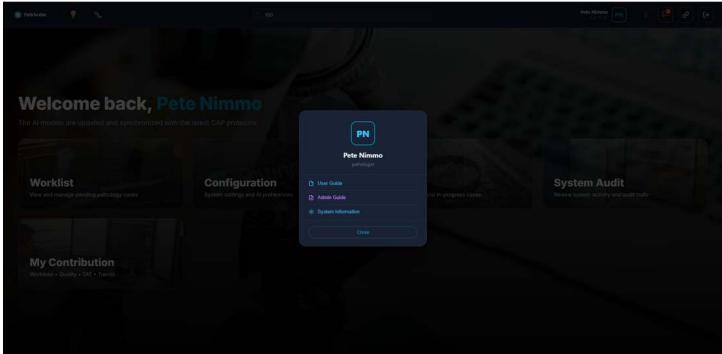
Area	Example commands
Navigation	Open worklist / Open search / Open messages / Go home / Go back
Case navigation	Next case / Previous case / Open case [accession number]
Synoptic	Accept suggestion / Reject suggestion / Accept all suggestions / Next field / Save report
Finalisation	Finalise report / Submit for review
Messaging	Open messages / Compose message / Send message / Mark read
Search	Search cases / Clear filters / Export results
System	Open configuration / Sign out / Open voice settings

TIP

Use Chrome or Edge. Ensure your voice profile (EN-US or EN-GB) is set correctly in your profile settings. Speak clearly after the activation tone. PathScribe recognises 139 registered actions and learns unrecognised phrases permanently.

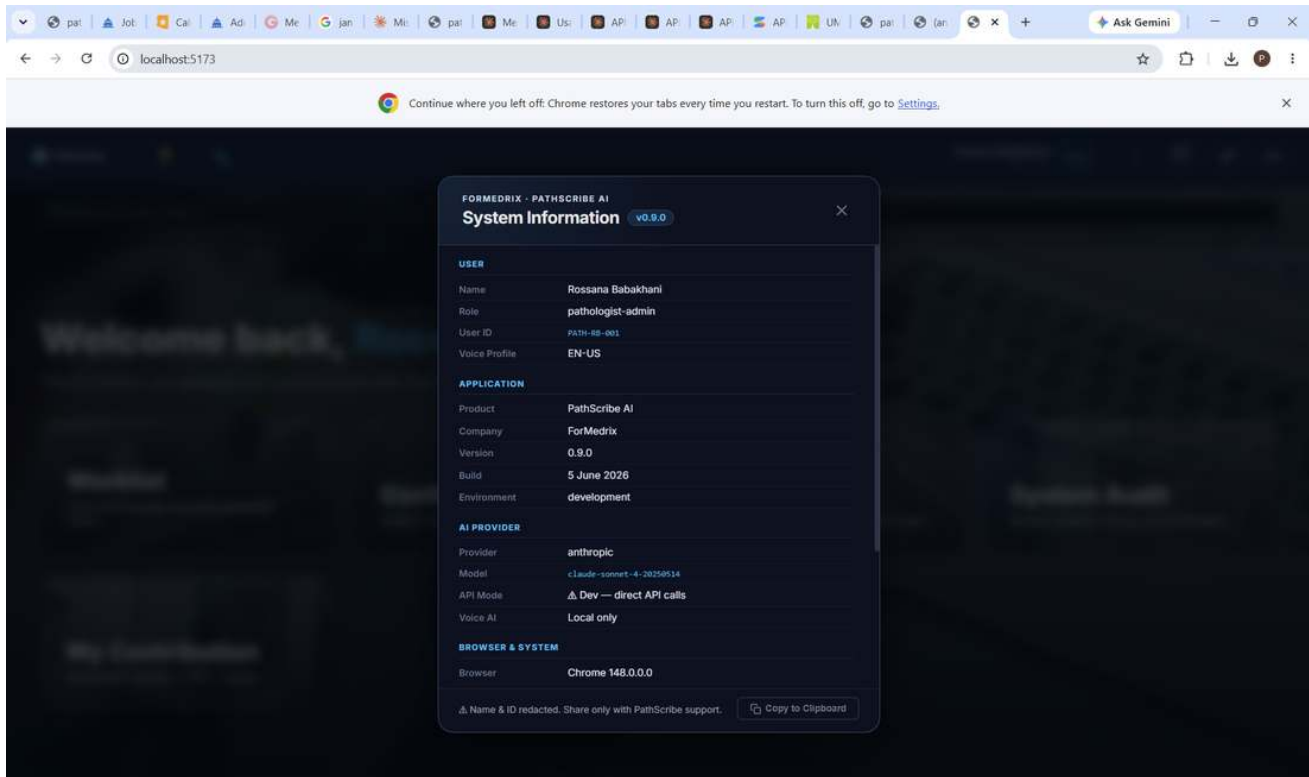
14. Your Profile

Click your name or initials badge (top right) to open the profile panel.



Profile panel -- User Guide, Admin Guide, System Information.

Option	Details
User Guide	Opens this document in a new browser tab.
Admin Guide	Opens the Admin Guide (visible to Admin and Pathologist-Admin roles only).
System Information	Application version, AI provider and model, API connectivity status, browser and OS details.
Copy to Clipboard	Generates a formatted support report. Name and ID are redacted -- safe to share with PathScribe support.



System Information -- User, Application, AI Provider, and Browser & System sections.

KNOWN LIMITATION (as of this writing)

This screenshot's AI Provider section shows Model: claude-sonnet-4-20250514 and API Mode: Dev -- direct API calls. Both are environments/values the Admin Guide flags as needing correction before go-live (stale model ID, Section 6; production AI proxy required, Pre-Launch Checklist item 4) -- this is direct evidence the underlying issue is real, not just a documentation concern.

16. Validation Studies

Your institution may run a Validation Study -- a parallel-run comparison of AI-assisted reporting against existing workflow, used to measure how often AI-drafted narrative sections are accepted unchanged versus edited.

ADVISORY MODE

AI output is never submitted to the LIS without your review and approval, with or without an active study. Being enrolled in a study does not change how you work a case -- review and sign off exactly as you normally would.

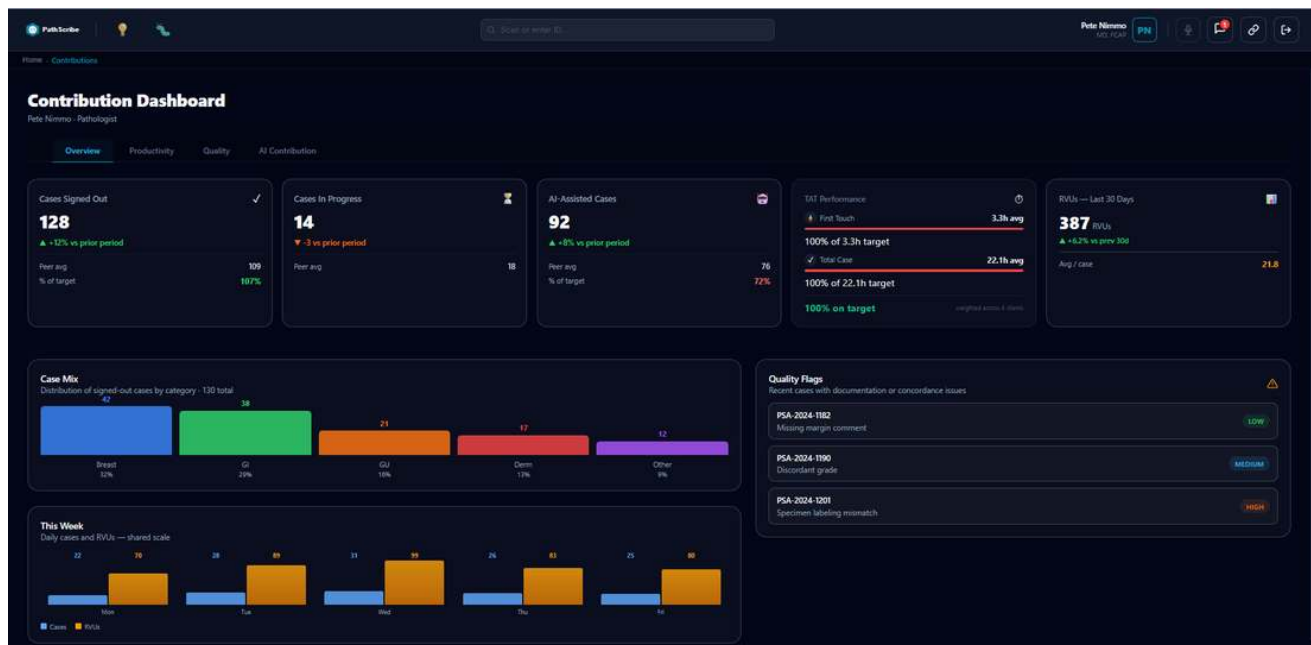
Validation Studies are configured by administrators (Configuration -> Validation Studies) -- see the Admin Guide, Section 19, for study creation, dashboards, and reports. If you're a lead pathologist with Admin access and need to set one up, that's where to look.

17. My Contribution

Home -> My Contribution opens your personal productivity, quality, and TAT dashboard, with four tabs: Overview, Productivity, Quality, and AI Contribution.

17.1 Overview

At a glance: Cases Signed Out, Cases In Progress, and AI-Assisted Cases, each compared against the prior period and your peer average. TAT Performance shows First Touch and Total Case time against their targets. RVUs (last 30 days) and a Case Mix breakdown by subspecialty are shown alongside a This Week daily cases/RVU chart.

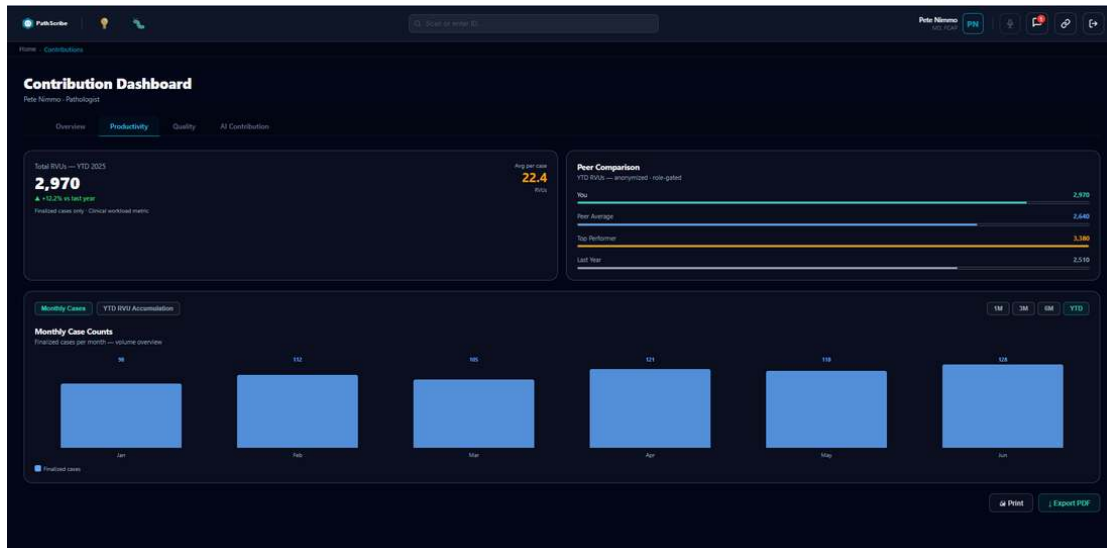


Contribution Dashboard -- Overview tab.

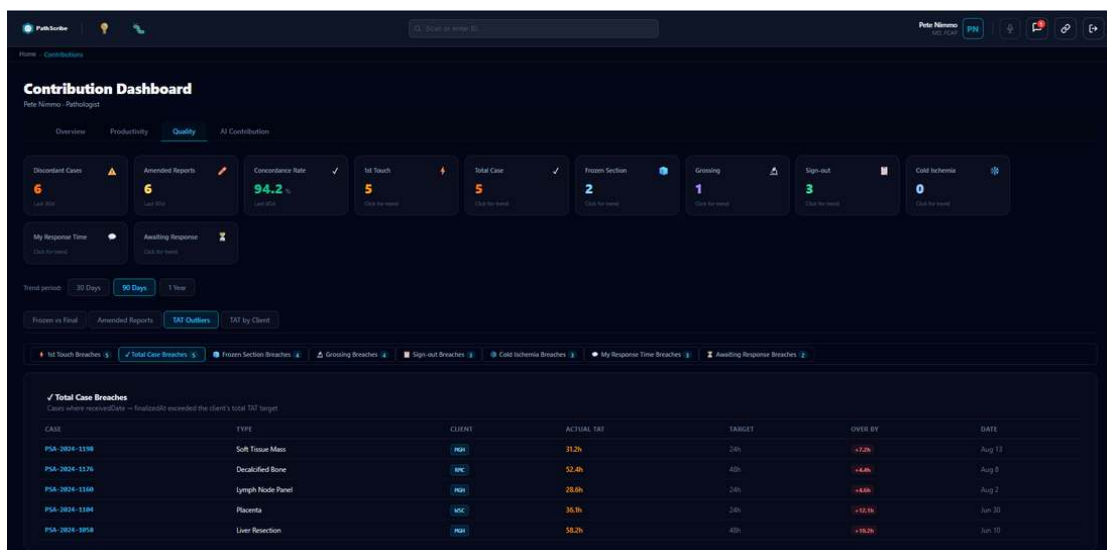
Quality Flags on this tab surface your own recent cases with documentation or concordance issues, each with a severity badge (Section A.4) -- e.g. a missing margin comment, a discordant grade, or a specimen labeling mismatch.

17.2 Productivity, Quality, and AI Contribution

The remaining three tabs break the same underlying metrics down further: Productivity (workload trends over time), Quality (concordance and documentation metrics in detail), and AI Contribution (how much of your signed-out content originated from AI suggestions versus your own authoring).



Productivity tab.



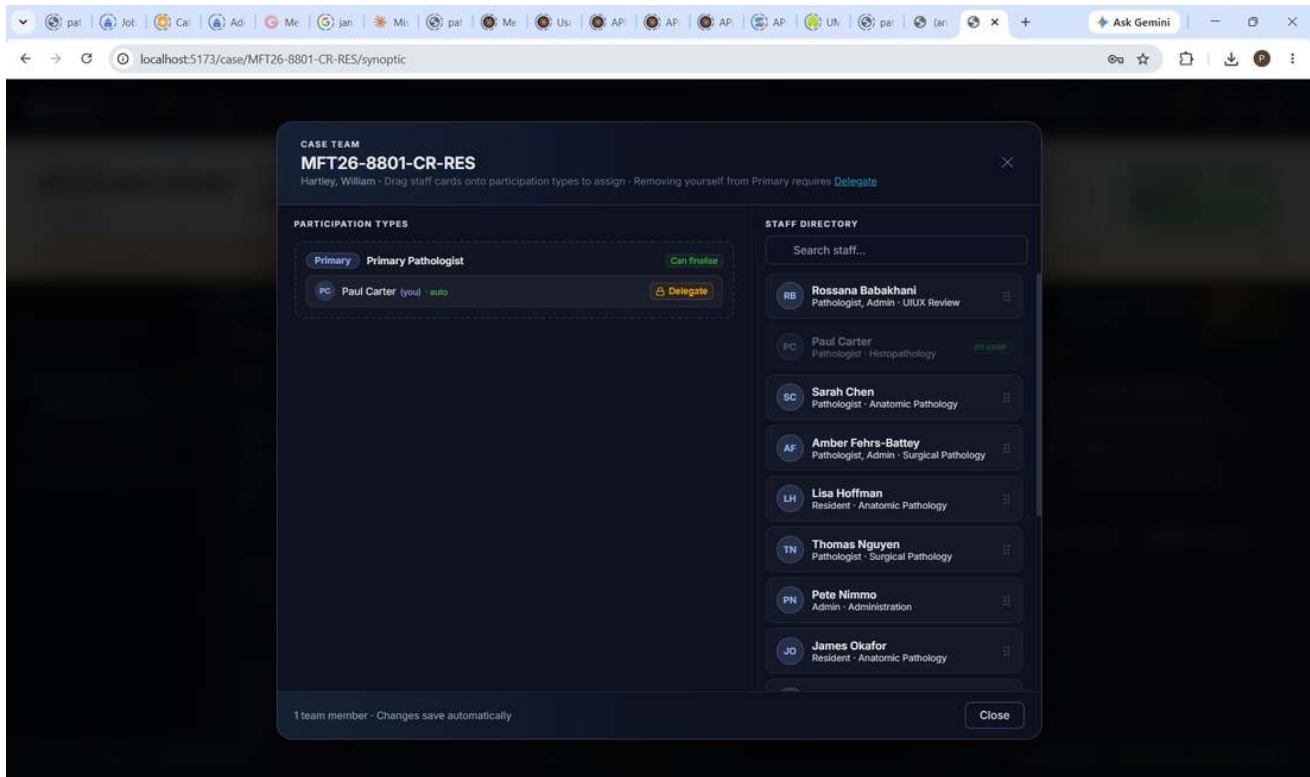
Quality tab.

18. Additional Case Tools

Several further tools are available from within an open case, beyond the diagnostic and synoptic workflow covered in Sections 5-7.

18.1 Case Team

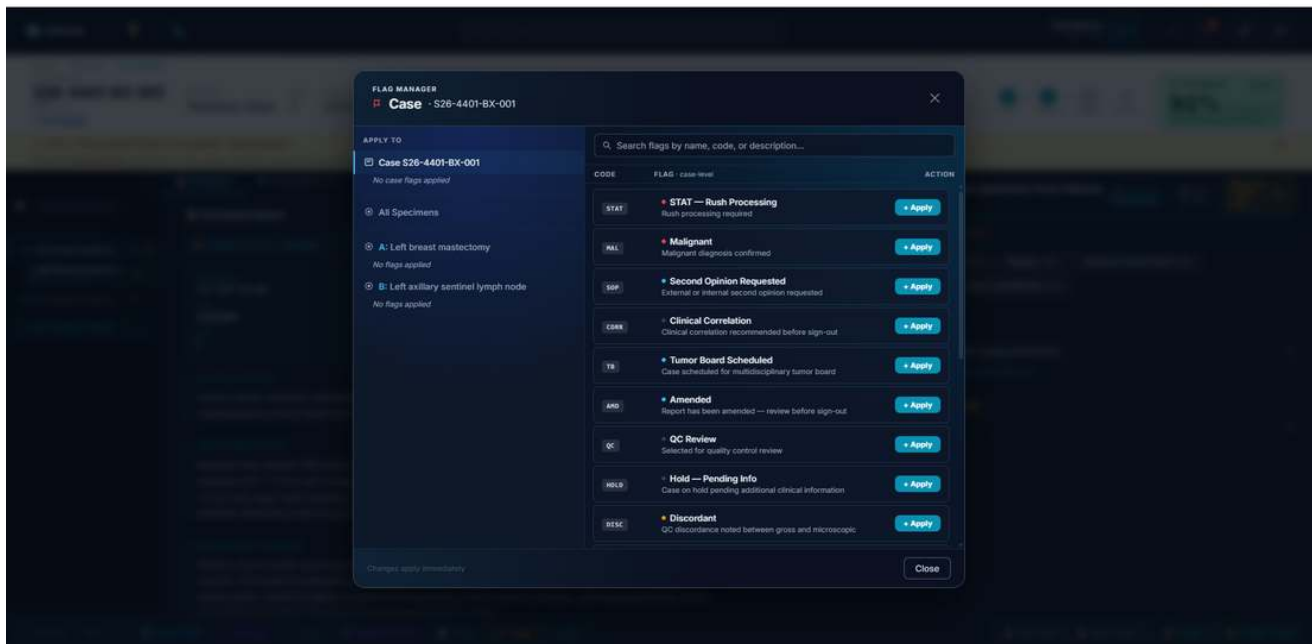
Click Team in the case header to open Case Team. Drag staff cards from the Staff Directory onto a Participation Type slot (Section 18.5 of the Admin Guide) to assign them to this case. Removing yourself from the Primary slot requires using Delegate (Section 10) rather than a direct removal.



Case Team -- drag staff onto Participation Type slots.

18.2 Flag Manager

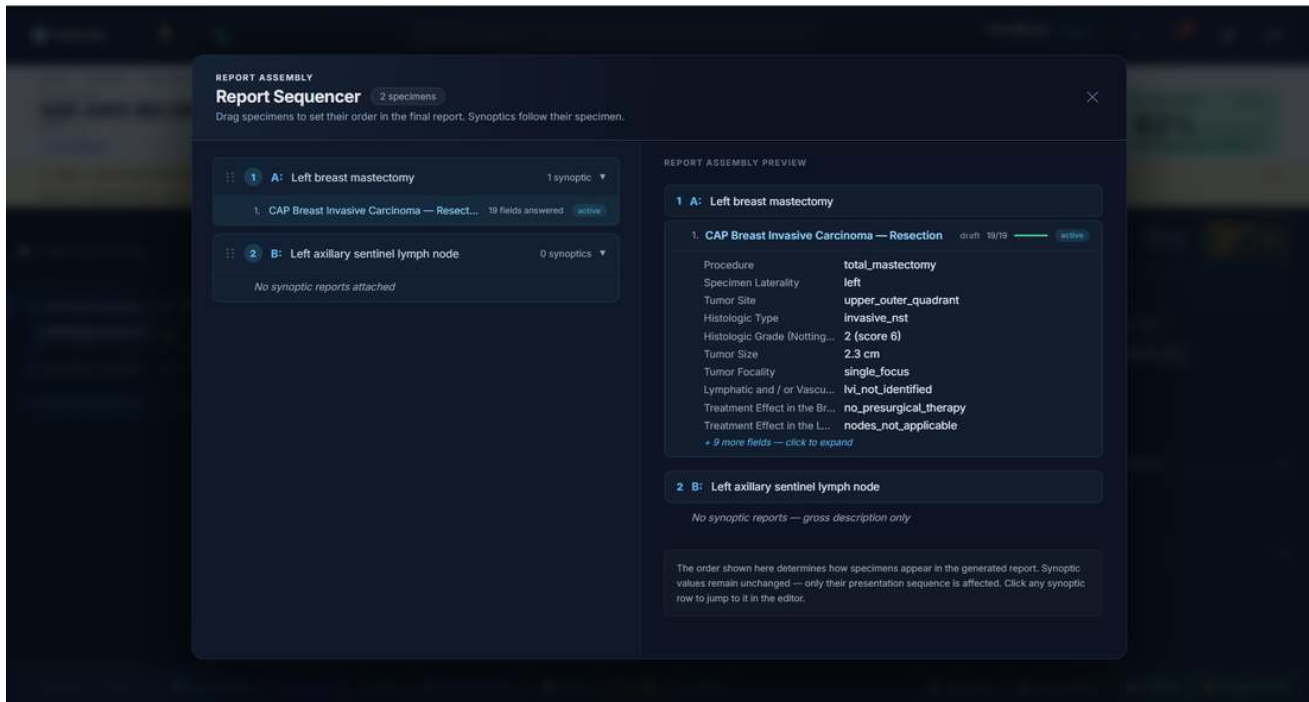
Click the flag icon in the case header to open Flag Manager. Apply flags at the case level or to an individual specimen -- the same flags defined in Configuration -> System -> Flags (Admin Guide Section 5). Changes apply immediately.



Flag Manager -- apply to case or any individual specimen.

18.3 Report Sequencer

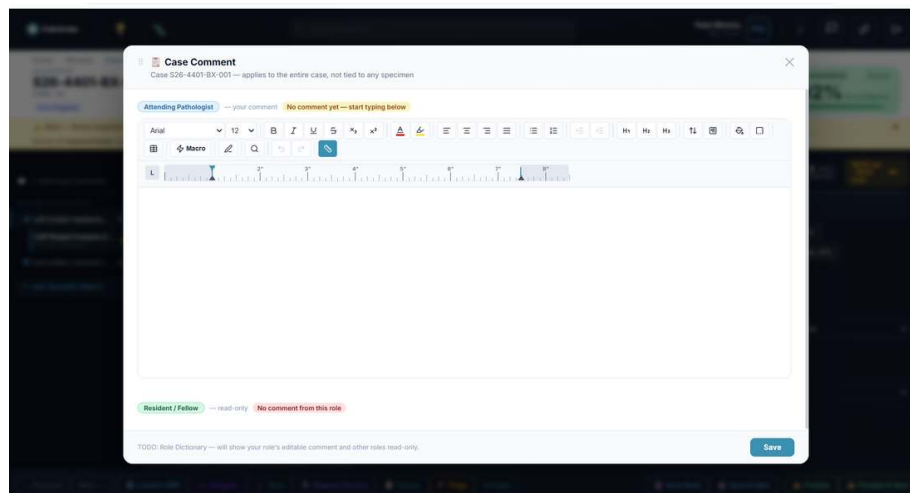
For multi-specimen cases, Report Sequencer lets you drag specimens (and their attached synoptic reports) into the order they should appear in the final report, with a live Report Assembly Preview alongside. This only affects presentation order -- synoptic values themselves are unchanged. Click any row in the preview to jump to it in the editor.



Report Sequencer -- drag to reorder specimens, with a live assembly preview.

18.4 Comments

Comments can be added at the case level (visible across all specimens) or at the specimen level (scoped to one specimen only), via the Comment control in the relevant panel.



Case-level comment.

18.5 Internal Notes

Internal Notes are for lab-internal communication about a case and are never included in the generated report or transmitted to the LIS -- distinct from Comments (Section 18.4), which can appear in report output depending on configuration. Note Type: Informal Review, Clinical Observation, Consultation, Addendum Request, or Other. Visibility: Shared (visible to the whole team) or Private.

PathScribe

Home Worklist Case Report

ACCESSION
S26-4401-BX-001

PATIENT
Thompson, Grace

SEX
F

DATE OF BIRTH
3/14/1974

MRN
100001

REFERRING
Metro General Hospital

Alert -- Some required fields are incomplete. [Click to review](#) --

Review all required fields marked with * in the synoptic checklist. Ensure all required data elements are completed before finalizing.

Full Patient Report

Received from US -- read-only

ACCESSION
S26-4401-BX-001

PATIENT
Thompson, Grace

DOB
3/14/1974

MRN
100001

SEX
F

PRIORITY
Routine

CLINICAL HISTORY

Invasive ductal carcinoma, left breast 10 o'clock, ER+/PR+/HER2 2+. Proceeding to mastectomy following multidisciplinary tumour board recommendation.

GROSS DESCRIPTION

Received fresh labeled "left breast mastectomy" is a 487g specimen, 18.0 x 14.0 x 4.5 cm. The overlying skin ellipse measures 18.0 x 7.0 cm and is unremarkable. Sectioning reveals a firm, stellate, tan-white mass measuring 2.3 x 1.8 x 1.5 cm in the upper outer quadrant, 3.0 cm from the nipple and 2.0 cm from the deep margin. No satellite nodules identified. Remaining breast tissue is fibrofatty.

Protocol for the Examination of Specimens from Patients with Carcinoma of the Breast

Jump to: [Next Unanswered \(16\)](#)

Specimen

Procedure: **Total mastectomy (including nipple)** **77%** **Confirm**

Specimen Laterality: **Left** **98%** **Confirm**

Left

Internal Notes

NOTE TYPE

Informal Review Clinical Observation

Consultation Addendum Request

Other

NOTE

Enter your clinical note -- avoid patient names, DOB, or MRN

These notes are part of the clinical record but will not appear in the formatted patient report.

VISIBILITY

Shared Private

Cancel Save Note

No internal notes yet for this case

[Previous](#) [Next](#) [Launch EMR](#) [Delegates](#) [Team](#) [Request Review](#) [History](#) [Flags](#) [Codes](#)

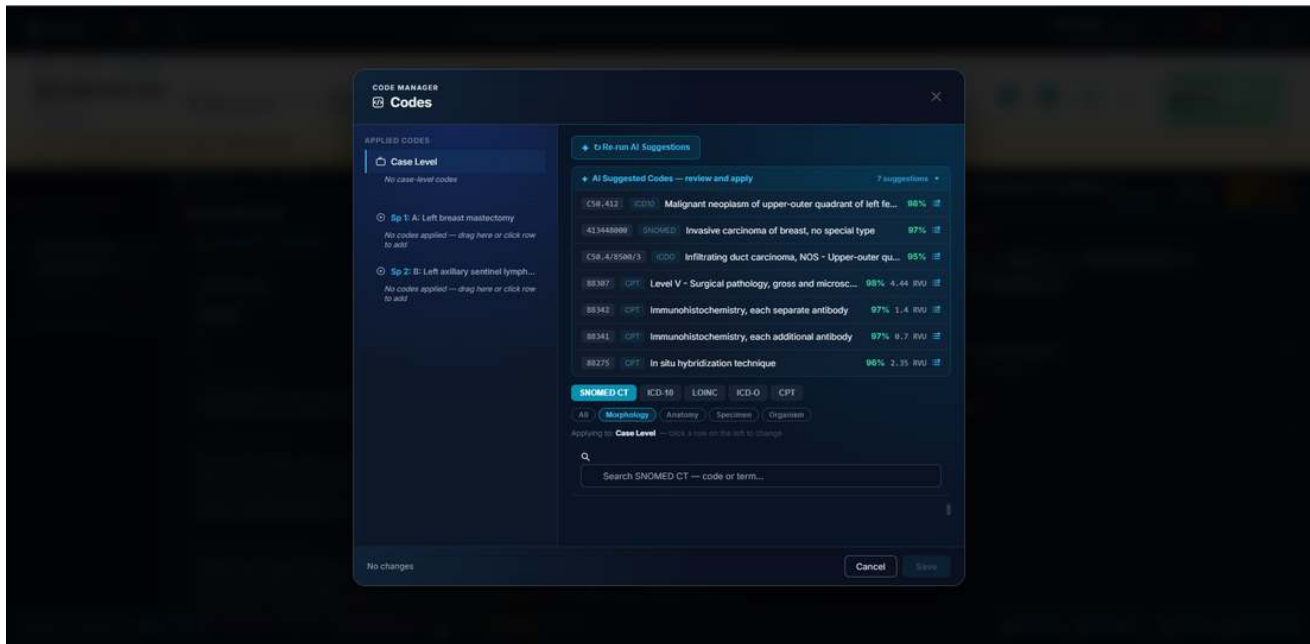
Internal Notes -- "Internal use only" banner, Note Type, PHI-safety placeholder, Visibility.

NOTE

The note field placeholder explicitly warns: avoid patient names, DOB, or MRN -- same PHI-safety pattern used when messaging a pathologist from Patient History (Section 18.8).

18.6 Code Manager

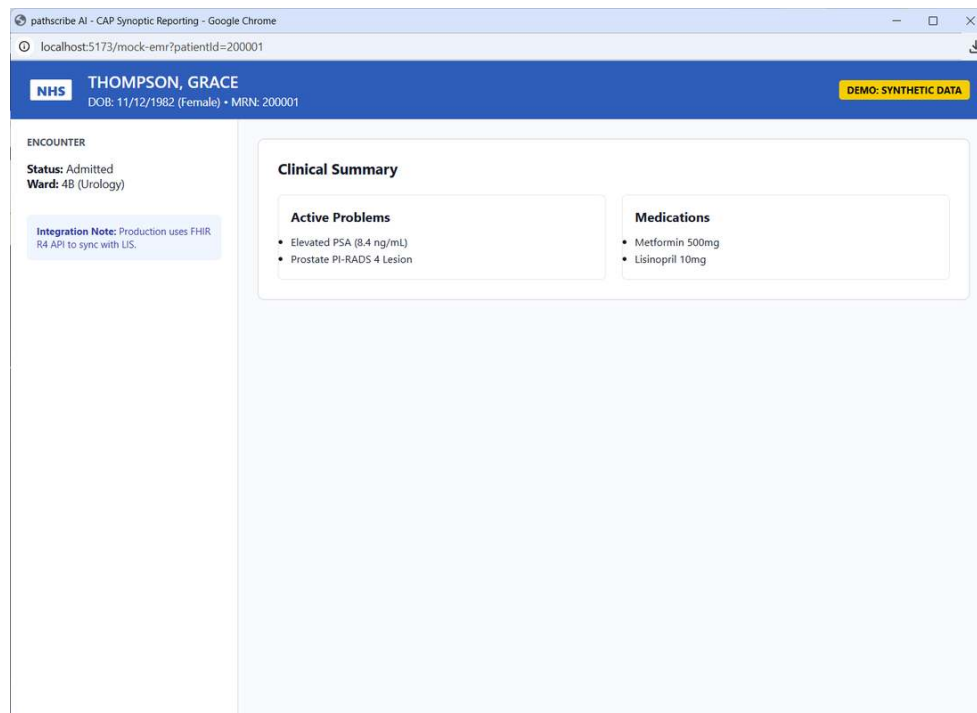
Opens the full coding interface for a case -- the detailed view behind the Coding panel introduced in Section 6.4, for reviewing and managing every assigned ICD-10 and SNOMED CT code at once.



Code Manager -- full coding interface.

18.7 Launch EMR

Opens the patient's record in the connected EMR/EHR system in a new tab, for clinical context beyond what PathScribe itself holds (active problems, current medications, encounter status).



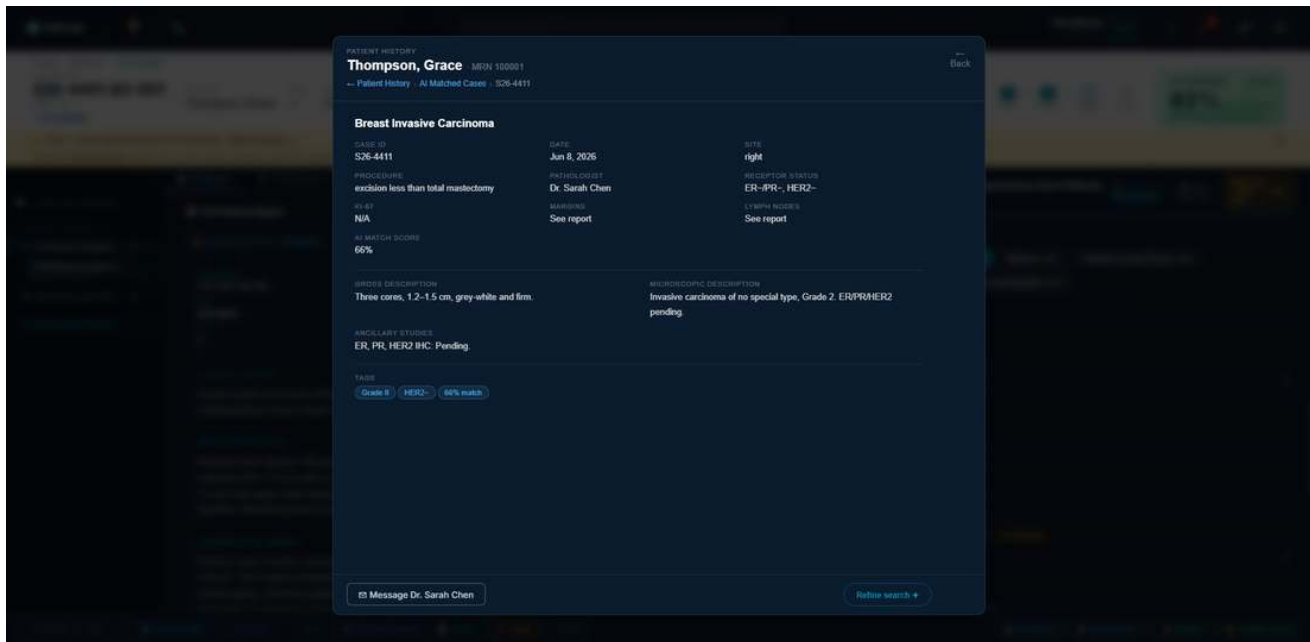
Launch EMR -- patient encounter, active problems, and medications.

NOTE

This screenshot shows the demo/synthetic-data EMR view used in non-production environments. Production deployments sync via a FHIR R4 API to your real LIS/EMR.

18.8 Patient History

Beyond the basic Patient History link (Section 4.2), the expanded view supports a refined search and can surface an AI-matched prior case for comparison, with the option to message the prior case's pathologist directly.



Patient History -- AI-matched prior case.

Refine search reuses the full Case Search filter set (Section 11) -- reached via Patient History -> Case Search, with a 1-year lookback applied by default.

Patient History -- Refine Search (the same filters as Section 11.2).

Selecting a prior case opens its full record -- diagnosis, gross/microscopic findings, receptor status, and pathologist comment -- with an option to message that case's pathologist directly.

Prior case detail, with a message field to the original pathologist.

PHI SAFETY

The message field explicitly warns not to include patient name, MRN, or diagnosis -- this channel is for clinical correlation discussion, not for transmitting identifiable patient data.

19. Audit Log

Home -> System Audit opens System Logs -- a complete record of AI actions, user changes, system events, and errors. Four summary counters: Total Events, AI Actions, User Changes, and System Events. Filter by type (All / AI / User / System), by user, by date range, or search by event, detail, or case ID.

TIMESTAMP	TYPE	EVENT	DETAIL	USER	CASE ID
2024-06-25 09:16:11	User	MESSAGE DELETED	Message moved to Recently Deleted -- Subject: "Message" from Pete Nimmo	Dr. Sarah Li Chen	
2024-06-25 09:17:04	User	MESSAGE SENT	Message sent to Dr. Sarah Li Chen -- Subject: "Message" [URGENT]	Pete Nimmo	
2024-06-25 09:17:05	User	MESSAGE SENT	Message sent to System Admin -- Subject: "Message" [URGENT]	Pete Nimmo	
2024-06-26 20:42:00	AI	Synoptic Generated	AI generated synoptic v1 using CAP Breast Invasive protocol	System (AI)	1208-4401
2024-06-26 20:43:00	User	Field Updated	Synoptic field "Tumour Size" updated	Dr. Sarah Johnson	1208-4401
2024-06-26 20:43:00	User	Synoptic Finalized	Synoptic report finalized and signed	Dr. Sarah Johnson	1208-4401
2024-06-26 21:13:00	AI	Protocol Selected	AI auto-selected CAP Colon Resection protocol	System (AI)	1208-4402
2024-06-26 21:20:00	System	LIS Sync	Microscopic description received from LIS	System (LIS)	1208-4402
2024-06-26 21:53:00	AI	Synoptic Updated	AI updated synoptic v2 after microscopic findings ingested	System (AI)	1208-4400
2024-06-26 22:10:00	System	User Login	User authenticated successfully	Dr. Sarah Johnson	
2024-06-26 22:10:00	System	User Login	User authenticated successfully	Dr. Michael Chen	
2024-06-26 22:10:00	AI	Protocol Selected	AI auto-selected CAP Lung Resection protocol	System (AI)	1208-4403
2024-06-26 22:50:00	System	LIS Sync	Gross description received from LIS	System (LIS)	1208-4403
2024-06-26 22:52:00	AI	Synoptic Generated	AI generated synoptic v1 using CAP Lung Resection protocol	System (AI)	1208-4399
2024-06-26 23:16:00	User	Field Updated	Synoptic field "Histologic Type" updated	Dr. Emily Rodriguez	1208-4398
2024-06-26 23:22:00	User	Synoptic Finalized	Synoptic report finalized and signed	Dr. Emily Rodriguez	1208-4398
2024-06-26 23:23:00	System	LIS Sync	Gross description received from LIS	System (LIS)	1208-4398
2024-06-26 23:30:00	User	Protocol Changed	Protocol changed from CAP Prostate Biopsy to CAP Prostatectomy	Dr. Michael Chen	1208-4396
2024-06-26 23:30:00	AI	Protocol Selected	AI auto-selected CAP Thyroid protocol	System (AI)	1208-4397
2024-06-26 23:30:00	System	Comment Added	Endoscopic ultrasound procedure complete 4/26	Dr. Sarah Li Chen	1208-4394

System Logs -- AI/User/System events with case-ID links, filterable and exportable.

Each row links directly to its case (where applicable) so you can jump straight from an audit event to the case it concerns. A separate **Error Log** tab (next to Audit Log) surfaces system errors specifically. Click Export CSV for compliance reporting.

NOTE

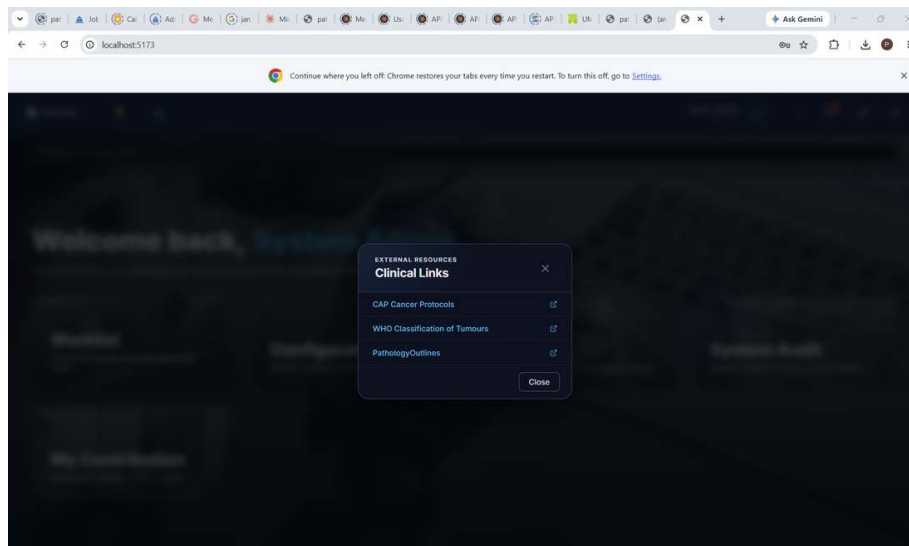
Event types you'll see here include AI actions (Synoptic Generated, Protocol Selected, Synoptic Updated), user actions (Field Updated, Synoptic Finalized, Protocol Changed, Comment Added, messages sent/deleted), and system events (LIS Sync, User Login) -- consistent with the audit event categories described in the Admin Guide, Section 9.

20. Top Navigation Bar -- Additional Elements

A few navigation bar icons (Section 1.3) open their own dedicated tools.

20.1 Clinical Links

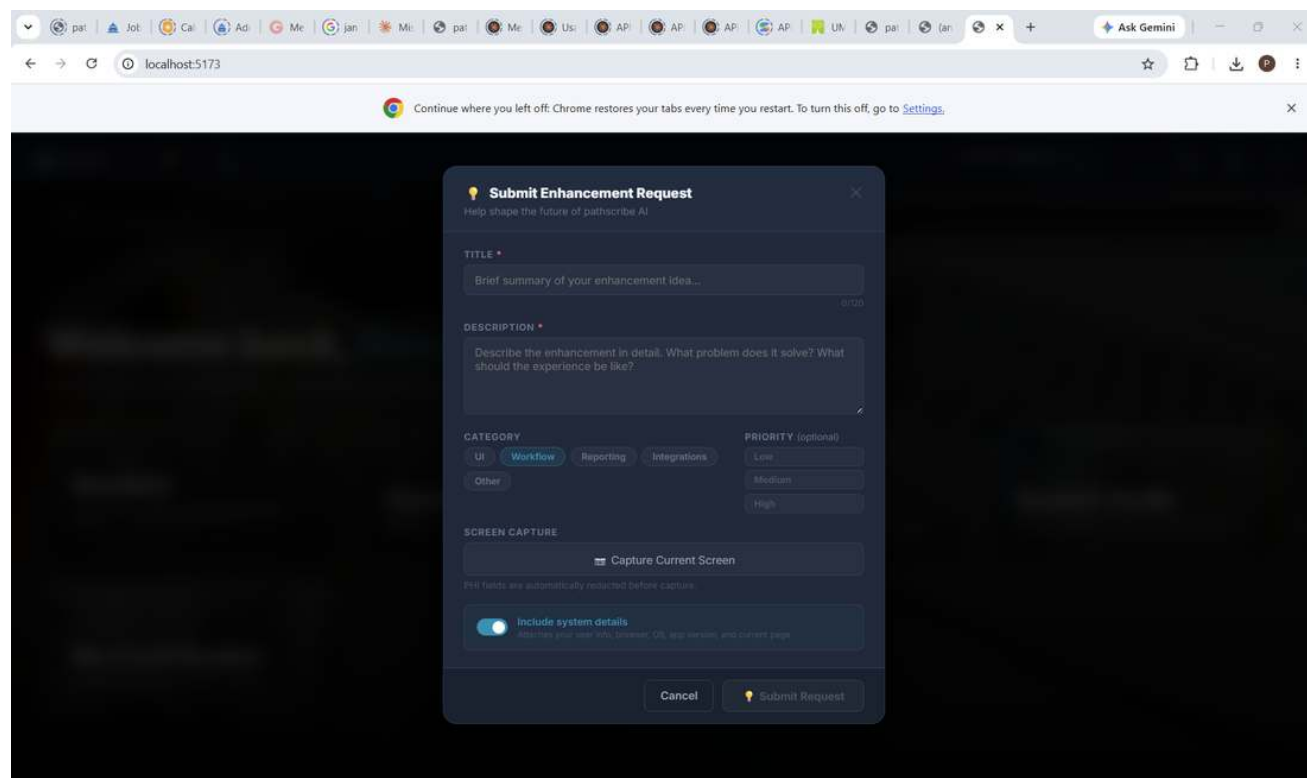
Click the lightbulb icon for quick access to external clinical references: CAP Cancer Protocols, WHO Classification of Tumours, and PathologyOutlines. Each opens in a new tab.



Clinical Links -- external reference shortcuts.

20.2 Enhancement Request

Submit a feature idea: Title, Description, Category (UI, Workflow, Reporting, Integrations, Other), and optional Priority. Capture Current Screen attaches a screenshot with PHI fields automatically redacted; Include System Details attaches your user info, browser, OS, app version, and current page.



The screenshot shows a web browser window with the address bar displaying 'localhost:5173'. A notification bar at the top states: 'Continue where you left off: Chrome restores your tabs every time you restart. To turn this off, go to [Settings](#).' Below this, a dark-themed modal window titled 'Submit Enhancement Request' is open. The modal has a close button in the top right corner. Inside the modal, the text 'Help shape the future of pathscribe AI' is displayed. The form contains the following fields and controls:

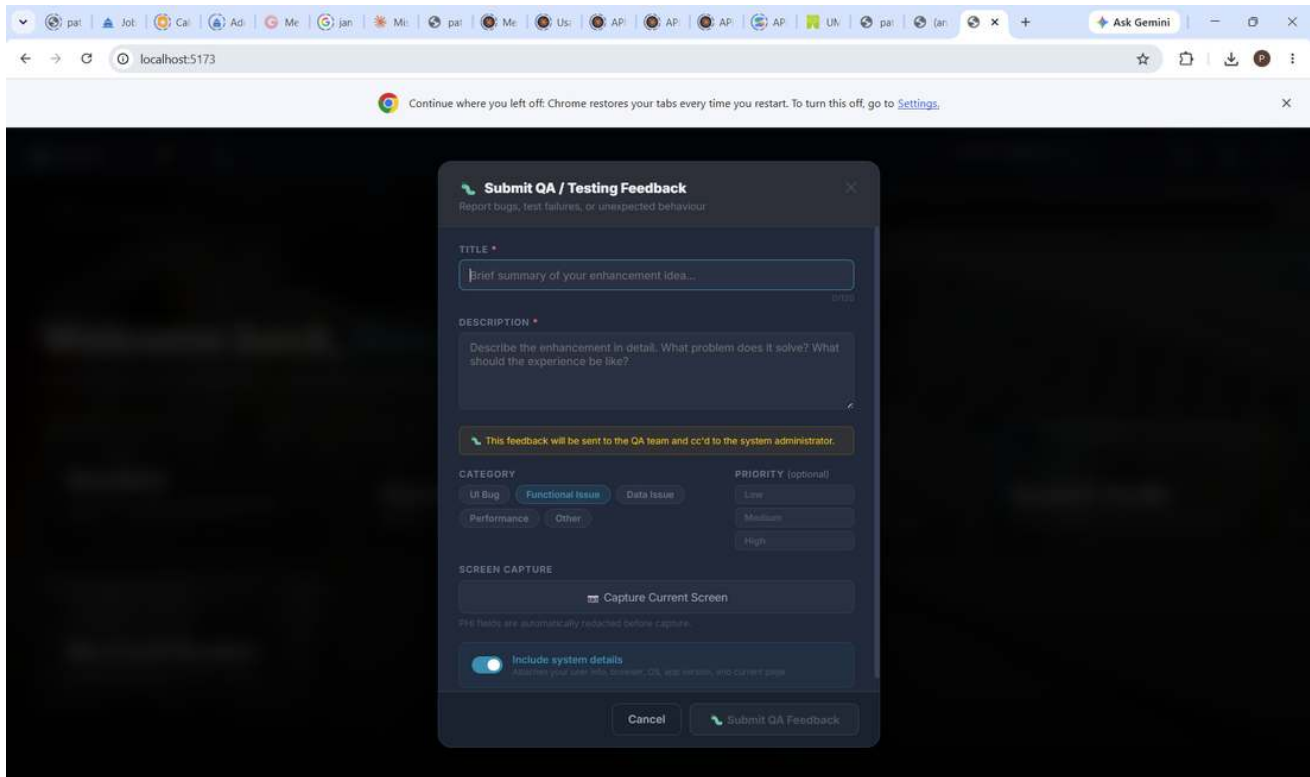
- TITLE ***: A text input field with the placeholder 'Brief summary of your enhancement idea...'. A character count '0/100' is shown to the right.
- DESCRIPTION ***: A larger text input field with the placeholder 'Describe the enhancement in detail. What problem does it solve? What should the experience be like?'. A character count '0/1000' is shown to the right.
- CATEGORY**: A group of buttons: 'UI', 'Workflow' (selected), 'Reporting', 'Integrations', and 'Other'.
- PRIORITY (optional)**: A group of buttons: 'Low', 'Medium', and 'High'.
- SCREEN CAPTURE**: A button labeled 'Capture Current Screen' with a camera icon.
- Include system details**: A toggle switch that is currently turned on. Below it, small text reads: 'Receives your user info, browser OS, app version, and current page'.

At the bottom of the modal are two buttons: 'Cancel' and 'Submit Request' (which has a lightbulb icon).

Submit Enhancement Request.

20.3 QA / Testing Feedback

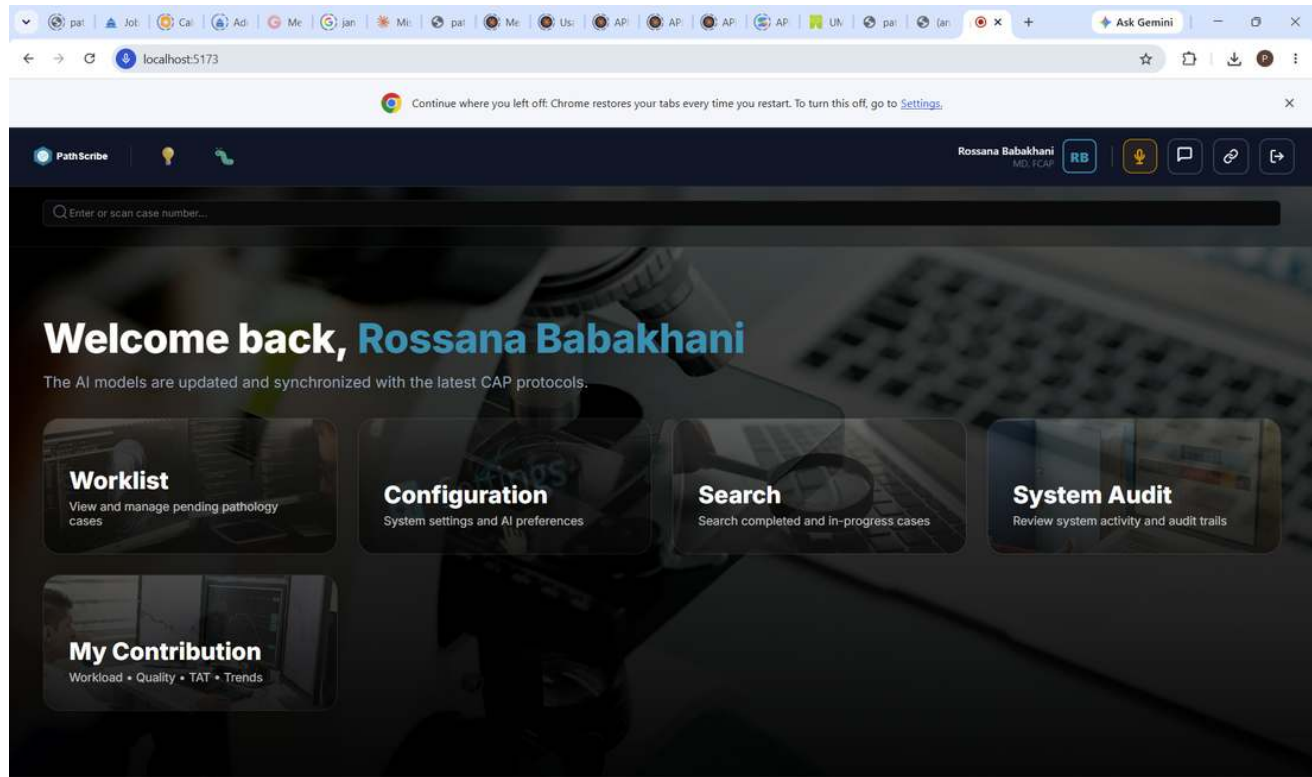
Distinct from an Enhancement Request -- this is for reporting bugs, test failures, or unexpected behaviour, not feature ideas. Category options reflect that: UI Bug, Functional Issue, Data Issue, Performance, Other. Feedback is sent to the QA team and cc'd to the system administrator. Same screen-capture and PHI-redaction behaviour as Enhancement Request.



Submit QA / Testing Feedback.

20.4 Voice Toggle Indicator

The microphone icon in the navigation bar turns solid orange/red while actively listening, so you have a clear at-a-glance indicator of whether voice input is live.



Active voice listening indicator.

Quick Reference

Action	How
Return to Home	Click the PathScribe by ForMedrix AI logo (top left)
Open Worklist	Home -> Worklist tile / voice: "open worklist"
Open a case	Click any worklist row
Navigate specimens	Click specimen tabs A, B, C... in the right panel
View patient history	Patient History link in the left panel
Accept AI suggestion	Click tick next to the suggestion
Accept all suggestions	"Accept All" button at the top of the synoptic panel
Review protocol change	Protocol Change Review modal -- appears automatically after microscopic is saved (v0.9.0)
View computational results	Click Computational tab in the synoptic panel
Order an additional test	+ Order additional test at the bottom of the Computational navigator
Preview report	Preview Report button in the case header
Finalise a case	Finalise button -> 3-step gate -> Pre-Finalisation Review -> sign
Reorder synoptics before sign-out	Drag specimens in the Pre-Finalisation Review left pane (v0.9.0)
Exclude a synoptic from transmission	Click Exclude in the Pre-Finalisation Review left pane (v0.9.0)

Action	How
Delegate a case	Delegate button in the case header
Request informal review	Request Review option in the case header
Claim a pool case	Pool Cases tile -> click a pool case row -> Claim
Search all cases	Home -> Search tile
Send a message	Nav bar speech bubble -> compose icon
Get help	Your initials badge (top right) -> User Guide
Support report	Your initials badge -> System Information -> Copy to Clipboard

Appendix A -- Colour & Visual Language Reference

This appendix explains the meaning of every colour, icon shape, and status indicator used across PathScribe.

A.1 Case Status Indicators (Worklist dots)

Colour	Meaning
Red -- STAT / Urgent	Case flagged STAT or has a pending urgent action. Requires immediate attention.
Amber -- Action Required	Needs user input: outstanding query, delegation pending, or a required field is missing.
Blue -- In Progress	Actively being worked on (grossing, microscopy, or synoptic in draft).
Green -- Complete / Signed Out	Report finalised and countersigned where required.
Grey -- Awaiting Micro / Grossed	Specimen received and grossed; sections processing; no pathologist action yet required.
Purple -- Delegated	Case delegated to another pathologist or in a review pool.

A.2 Case Flags

Colour	Flag class	Examples
Teal	Administrative	RUSH, Frozen Section, Tumour Board, MDT Discussion, QI Review
Cyan	Computational	IHC panels, molecular assays, BRAF, PD-L1, ER/PR/HER2
Amber	Quality / Review	Amended Report, Addendum Added, Correlation Required, Second Opinion
Green	Positive / Complete	AI coding applied, countersign received, research consent captured
Red	High-severity	Critical findings or urgent process flags

A.3 TAT Progress Bars

Colour	% of SLA consumed	Action
Green	< 70%	Well within the SLA. No intervention needed.
Amber	70-90%	Approaching the SLA limit. Monitor closely.
Red	> 90%	At or exceeding the SLA. Escalation or prioritisation required.

A.4 Severity Badges (Quality Tab)

Badge	Meaning
LOW (green)	Minor documentation issue or concordant variance. No immediate escalation required.
MEDIUM (amber)	Moderate discordance (e.g. grade upgrade) or process deviation. Discuss at QI review.
HIGH (red)	Significant diagnostic discordance or patient-safety relevant amendment. Requires formal root-cause analysis.

Index

Accept All 11
Accession Date 9
AI Copilot 11
Ancillary Studies 10
Audit Log 35
Biometric Setup 6
Case Delegation 16
Case Flags 41
Case Search 16
Case Status Indicators 41
Case Team 27
Clinical Links 6, 35
Code Manager 31
Coding 12
Comments 30
Computational Sidecar 12
Computational Tab 12
Delegated Cases 10
Delegation Type 16
Enhancement Request 6, 36
Error Log 35
External Resources 35
Finalise 15
Flag Manager 28
Gross Description 10
Internal Messaging 18
Internal Notes 30
Launch EMR 32
Logging In 6
Messages 18
Microscopic Description 10
My Contribution 6, 25
Navigation Bar 6
Patient History 10, 33
Pool Cases 8
Pre-Finalisation Review 15
Preview Report 15
Protocol Change Review 11
QA Enhancement 37
Report Sequencer 29
Request Informal Review 22
Search Filters 17
Severity Badges 41
Smart Identifier Box 17
SNOMED CT 12
Sorting 9
Status Tiles 8
Synoptic Reporting 11
System Audit 6
System Logs 35
TAT Progress Bars 41
Validation Studies 25
Voice Commands 23
Worklist 7